

Emergency ready primary health care operational guide



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Foreword

Health emergencies are no longer exceptional, they are constant, complex and increasingly interconnected. From infectious disease outbreaks to climate-related shocks, conflict and displacement, these threats are first detected – or missed – at the level of primary health care (PHC). The difference between rapid control and widespread harm depends on what happens at the frontline.

Yet PHC systems are too often not equipped to manage this reality.

The emergency ready primary health care (ERPHC) approach was developed to close this gap. Aligned with the World Health Organization's PHC vision and global health emergency frameworks, ERPHC strengthens four interconnected capacities – **connect, detect, protect and treat** – to ensure PHC systems can rapidly identify threats (speed), safeguard patients and health workers (safety) and maintain services under pressure (surge).

The framework which accompanies this operational guide defines the core capacities required for emergency ready PHC. The operational guide translates them into practical, adaptable steps for countries to contextualize, prioritize and implement according to their governance structures, risks and resources. Together, these documents support countries to embed emergency readiness into routine PHC operations – avoiding parallel systems and reinforcing essential public health functions, community engagement, surveillance, infection prevention and control, case management and service continuity.

Lessons from multi-country implementation reaffirm that when PHC facilities, communities and public health authorities work together, performance improves rapidly. Integrated workforce strengthening, reliable supplies, resilient infrastructure, functional surveillance and trusted community relationships are the foundation of effective, equitable and resilient health systems.

We call on governments, partners and communities to use this framework and operational guide to accelerate progress towards PHC systems that are ready to protect people before, during and after emergencies. Strengthening emergency ready PHC is essential to achieving health security, universal health coverage and resilient health systems.

We extend our appreciation to the many experts, country teams, frontline health workers and community partners whose experience, insights and leadership shaped the development of this work.

Together, we remain committed to supporting countries to build PHC systems that are not only accessible and high quality, but also ready – every day – to save lives and protect communities.



A handwritten signature in black ink, appearing to read 'Chikwe Ihekweazu', written over a white background.

Dr Chikwe Ihekweazu
Executive Director
WHO Health Emergencies Programme



A handwritten signature in black ink, appearing to read 'Thomas Frieden', written over a white background.

Dr Tom Frieden
President and CEO
Resolve to Save Lives

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This framework was developed as a collaboration between RTSL and WHO headquarters, Geneva, Switzerland; the lead writers were Lilian Kiapi (RTSL) and Kwang Rim (Community Protection and Resilience Unit, WHO headquarters).

From WHO headquarters, overall strategic and technical guidance was provided by Kai von Harbou (Unit Head, Community Protection and Resilience Unit) under the leadership of Stella Chungong, (Director, Department of Health Emergency Preparedness). Special thanks are due to Nedret Emiroglu (Director of former Department of Country Readiness Strengthening) for leading the initial discussions on this work and supporting the collaboration between the two organizations.

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All external experts submitted a declaration of interests to WHO, disclosing potential conflicts of interest that might affect or might reasonably be perceived to affect their objectivity and independence in relation to the subject matter of the meetings and guidance. WHO reviewed these declarations and concluded that none could give rise to a potential or reasonably perceived conflict of interest related to the subjects covered by the guidance.

Abbreviations

CHW	community health worker
EDRM	emergency and disaster risk management
ERPHC	emergency ready primary health care
HEPR	health emergency prevention, preparedness, response and resilience
IDSR	integrated disease surveillance and response
IPC	infection prevention and control
PHC	primary health care
PPE	personal protective equipment
RCCE	risk communication and community engagement
UHC	universal health coverage
WASH	water, sanitation and hygiene
WHO	World Health Organization

Glossary

The **7-1-7** target is a global health benchmark designed to improve epidemic preparedness by ensuring that any suspected disease outbreak is detected within 7 days, notified to authorities within 1 day, and met with early response actions completed within 7 days.

All-hazards approach is a comprehensive and integrated emergency preparedness framework that is adopted when planning response and mitigation measures for all hazards that may occur. This approach is a proactive way of planning and responding; it encompasses the full scope of emergencies, hazards or disasters that could most likely occur and impact communities, governments and organizations in their areas.

Anticipatory action is defined as acting ahead of a predicted hazardous event to prevent or reduce the impacts on lives, livelihoods and humanitarian needs before they fully unfold. This works best when activities, as well as trigger thresholds or decision-making rules, are pre-agreed and decisions are made to guarantee the fast release of pre-arranged funding (1).

Community event-based surveillance is the organized, early detection and reporting of ad hoc, unstructured information regarding potential public health risks by community members and volunteers. It focuses on identifying unusual health events, such as clusters of illness or sudden deaths, to enable rapid, local response before diseases spread, often complementing traditional, indicator-based, clinic-focused surveillance.

Community evidence refers to knowledge generated with or from local communities that reflects their experiences, perceptions, behaviours, needs and capacities in relation to a public health emergency or humanitarian crisis. It is distinguished from clinical or epidemiological evidence by its focus on the social, cultural and contextual dimensions of an emergency event and by the active or central role of local communities in its production (2).

Community health workers are a sub-set of health workers, individuals chosen by and accountable to the communities they serve, providing essential health services while acting as a bridge between communities and formal health systems. They are not professional, usually having less than two years of training and may be paid or volunteers.

Community protection refers to community-centred actions that safeguard the health, well-being and livelihoods of people at risk or affected by a health emergency. It emphasizes placing communities at the centre of decisions and actions, engaging them through risk communication, community engagement and infodemic management, implementing population and environmental public health interventions and coordinating multisectoral efforts to mitigate both health and broader social impacts of emergencies, with a strong focus on equity and accountability (3).

Data are basic, often unprocessed analogue or digital elements, characteristics or representations of facts or information. This includes raw and unprocessed numbers, measurements or text (4).

Disaster risk management is the systematic process of identifying, reducing and managing risks from hazards through prevention, preparedness, response and recovery measures to minimize their impacts on people, systems and services.

Emergency cycle describes the continuous process used to manage risks and respond to emergencies before, during and after an event. It is also called the disaster management cycle.

Emergency preparedness is the continuous process of planning, capacity building and strengthening systems to prevent, detect and respond to health emergencies.

Emergency readiness is the functional state of being ready to respond promptly, safely and effectively to health emergencies by executing the preparedness plans.

Essential public health functions are a set of fundamental, interconnected and interdependent activities required to ensure effective public health action to prevent disease, promote and protect health and well-being and address broad health determinants coming from environmental, social, cultural, lifestyle and biopsychosocial drivers.

Frontline health workers include facility-based providers, community health workers, environmental health workers, public health officers and those that work across health and other sectors. Frontline health workers are uniquely positioned to influence people's behaviours as trusted sources of health information and advice in communities and role models for the acceptance and uptake of protective measures during health emergencies.

Health emergency An acute event or hazard that threatens population health and requires urgent response to prevent or reduce harm (5).

Health emergency prevention, preparedness, response and resilience framework is the World Health Organization's comprehensive framework for strengthening global and national capacities to prevent, prepare for, detect and respond to health emergencies and to build resilience so health systems can withstand and recover from shocks.

Joint external evaluation is a voluntary, collaborative, multisectoral process to assess country capacities to prevent, detect and rapidly respond to public health risks whether occurring naturally or due to deliberate or accidental events

Medical countermeasures are the health products and technologies used to prevent, detect and respond to health emergencies and other serious public health threats, including vaccines, medicines (such as therapeutics and antivirals), diagnostics, personal protective equipment and other essential health tools that reduce the impact of outbreaks, pandemics and biological, chemical, radiological or other hazards.

Primary care refers to the clinical and service delivery functions of primary health care that provide first-contact, continuous, comprehensive and coordinated care to individuals and families.

Primary health care is an approach to build and strengthen health systems. It is a whole-of-society approach to health that aims to maximize the level and distribution of health and well-being through three components: (a) primary care and essential public health functions as the core of integrated health services; (b) multisectoral policy and action; and (c) empowered people and communities (6).

Public health emergencies are serious health events that poses a significant risk to population health and may overwhelm routine health system capacity, requiring extraordinary public health measures.

Rapid response teams are pre-trained, multidisciplinary teams for emergency and disaster risk management. Rapid response teams that can be rapidly deployed to detect, assess and respond to emergencies or health threats in order to contain hazards, support affected populations and prevent further impact.

Resilient infrastructure is defined as the ability of a health facility's physical structures, systems and supporting services to remain functional, accessible and safe during and immediately after emergencies and disasters (7).

Reverse triage is a strategic emergency management approach that prioritizes resources for the least injured or most stable patients to maximize survival rates, return personnel to duty or free up health facilities during disasters. Reverse triage helps redistribute resources by identifying the least critical patients who require fewer resources.

Risk communication and community engagement is a core public health function that enables authorities and communities to work together to deliver timely, trusted and actionable information, listen to community concerns and perceptions and support people and communities to make informed decisions and adopt protective behaviours before, during and after health emergencies.

States Parties Self-Assessment Annual Reporting Tool is a self-assessment tool for Member States to use for reporting annual International Health Regulations (2005) metrics.

Universal health coverage means that all people have access to the full range of quality health services they need, when and where they need them, without financial hardship.

Executive summary

Emergency ready primary health care (ERPHC) is a system-wide approach that strengthens primary health care (PHC) to prevent, detect and respond to health emergencies while maintaining essential services. It does so through four integrated capacities – **connect, detect, protect and treat** – which together enhance community engagement and risk communication, early warning and surveillance, safe and resilient service delivery and effective case management and service continuity. Together these enable coordinated and non-fragmented frontline and system performance. ERPHC is aligned with the PHC vision and emergency preparedness principles of the World Health Organization and supports both universal health coverage and health security through a single, integrated PHC platform.

The ERPHC operational guide translates the ERPHC framework into step-by-step actions that enable PHC systems to deliver faster responses (**speed**), safer care for communities and health workers (**safety**) and the ability to manage increased demand and service disruption (**surge**). The operational guide offers learning bites based on documented experiences from a four-country ERPHC pilot.

The operational guide helps countries to design and implement ERPHC through a flexible, non-prescriptive process adapted to local risks, system capacity and governance arrangements. Implementation begins with high-level engagement and stakeholder mapping, followed by a rapid analysis of hazards, vulnerabilities and PHC system readiness. National co-creation workshops then enable stakeholders to prioritise gaps, adapt technical options and develop a contextualised national ERPHC strategy and implementation roadmap.

ERPHC is **implemented as a coordinated, whole-system approach across national, subnational, facility and community levels**. To guide progressive systems strengthening, the operational guide introduces a structured country planning and prioritisation tool that supports countries to sequence actions, align investments and strengthen coordination and system integration over time.

Effective implementation depends on cross-cutting system enablers, including strong leadership and coordination, a trained and supported workforce, sustainable financing and procurement systems, interoperable information and surveillance platforms, resilient infrastructure and logistics, and strengthened digital health and data integration. The operational guide also provides practical guidance on implementation planning, support and monitoring.

Together, the ERPHC framework and operational guide provide countries with a coherent, integrated and scalable pathway to strengthen PHC as the first line of defence against all hazards—protecting communities and health workers, sustaining essential services and building resilient health systems before, during and after emergencies.

1. Introduction to the emergency ready primary health care (ERPHC) operational guide

The ERPHC operational guide provides practical, step-by-step guidance to support countries in translating the ERPHC framework into action at the primary health care (PHC) level. It outlines how to design, implement and monitor context-appropriate interventions that strengthen the capacity of PHC systems to prevent, detect and respond to health emergencies while maintaining essential services. The guide is intended for ministries of health and implementing partners, offering adaptable tools, processes and resources to support country-led implementation.

1.1. What is the ERPHC operational guide?

The ERPHC operational guide is a package of tools that accompanies the ERPHC Framework. This operational guide translates the all-hazards approach of the accompanying ERPHC framework, into practical, step-by-step actions for implementation of ERPHC. While different emergencies – such as conflict, outbreaks and climate-related disasters – place similar and often compounding pressures on health systems, the operational guide supports countries to use a single operational model to position PHC as the first line of defence. It provides concrete guidance, tools and examples to operationalize the **connect, detect, protect and treat** functions in routine practice, with adaptations by emergency type; these enable faster response, safer care for health workers and patients and more reliable surge and service continuity across diverse emergency contexts.

1.2 Purpose of the operational guide

The purpose of the ERPHC operational guide is to assist countries in operationalizing emergency readiness within PHC systems by translating the ERPHC framework into practical, actionable steps. It aims to guide ministries of health and implementing partners in planning, implementing and continuously improving interventions that strengthen prevention, detection, protection and response capacities at the PHC level.

The guide also promotes a structured, context-adaptable approach to implementation, ensuring that efforts are aligned with national systems and responsive to local needs.

1.3 Scope of the operational guide

The ERPHC operational guide applies across all levels of the health system, with a primary focus on the PHC platform, including community-based services, PHC facilities, frontline workers, managers and supporting multisector actors. It is designed to help national, subnational and district authorities plan and implement emergency ready PHC across the full emergency cycle of prevention, preparedness, response and recovery. The guide supports the integration of essential public health and clinical functions through the **connect, detect, protect and treat** package. It is non-prescriptive and adaptable to different governance arrangements, resource settings and fragile or climate-vulnerable contexts, while complementing existing health security, disaster risk management and PHC strengthening strategies.

The operational guide outlines the process for initiating ERPHC as an approach in a country. This includes:

- preliminary high-level engagement and alignment;
- stakeholder analysis and mapping;
- stakeholder engagement;
- country assessment, planning, prioritization and consultations;
- co-creation of implementation strategy at country level;
- development of country ERPHC strategy;
- resource identification and costing of interventions; and
- implementation planning, support and monitoring.

1.4 Lessons learnt from implementing ERPHC

The ERPHC approach has been piloted in nearly 1,000 health facilities in diverse settings in four countries (Ethiopia, Nigeria, Sierra Leone and Uganda), spanning private and public facilities, prison health facilities, urban areas and rural areas and small to large health facilities, with promising results. The ERPHC pilot demonstrated rapid and measurable improvements across multiple performance areas (8). Throughout this guide, dedicated ‘learning bites’ highlight practical insights from the implementation of the pilot, illustrating common challenges, operational adaptations and policy-relevant findings to inform future ERPHC scale-up.

1.5 Target audience for the ERPHC operational guide

The primary audience for the ERPHC operational guide is government decision-makers and policy-makers as well as national and subnational programme implementers responsible for translating policy into practice. This includes ministries of health – particularly units leading primary health care – emergency preparedness and response and health system strengthening, as well as provincial, regional and district health authorities, PHC managers and frontline implementers such as district health teams and facility-level leaders working in both routine and emergency contexts. The audience also includes ministries of disaster management, ministries of finance etc. as well as public health institutions.

Certain actions are relevant for broader audiences, including stakeholders at national and subnational levels such as community-based organizations; local, regional and international nongovernmental organizations; WHO regional and country offices; United Nations agencies; and humanitarian and development partners and donors involved in health emergency preparedness and response. In addition, the content is relevant to academic and training institutions involved in learning, capacity development and operational research to strengthen emergency ready primary health care.

1.6 How this document should be used

The ERPHC operational guide is intended to be used in conjunction with the ERPHC framework. Together, these two documents together provide a step-by-step approach for initiating ERPHC in a country. The operational guide is designed to be applicable across diverse settings and countries and therefore presents a range of possible actions, acknowledging that not all will be appropriate or prioritized in every context.

2. ERPHC implementation approach

The ERPHC operational guide provides a planning and implementation approach, rather than a fixed checklist or standard package; it offers a set of adaptable outputs that can be selected, sequenced and tailored based on local risks, system capacity and operational priorities. Countries and partners can contextualize the framework using this operational guide and structured risk and system assessments at national, subnational and facility levels. These assessments can be applied to identify hazards and define appropriate preparedness actions at PHC level.

Implementation of ERPHC begins with a rapid analysis of hazards, vulnerabilities and PHC system readiness that can be supported by using existing tools such as the Strategic Toolkit for Assessing Risks (9), with findings translated into context-specific actions for different levels of primary care. While the implementation approaches vary by context and PHC level, the framework's four core capacities (**connect, detect, protect and treat**) and the system outputs (**speed, safety and surge**) should remain constant and guide the planning and prioritization. Then, the delivery models, sequencing and operational methods are adapted to local realities through a co-creation process to ensure that ERPHC plans are feasible, implementable and aligned with priorities.

All implementation actions within this operational guide are designed to strengthen the three ERPHC system outputs: **speed** (timely detection and response), **safety** (protection of patients, communities and health workers) and **surge** (capacity to maintain and scale essential services during shocks). The performance objectives associated with these capacities and outputs guide prioritization, sequencing and resource allocation decisions throughout implementation.

2.1 Coordination and integration across capacities

The ERPHC implementation model applies a coordinated, systems-oriented and integrated approach; it aligns actions across the four core capacities – **connect, detect, protect and treat** – to avoid fragmentation and strengthen overall emergency readiness. Instead of implementing isolated or vertical activities, ERPHC brings together existing preparedness and response interventions across national, subnational, facility and community levels to deliver unified, outcome-driven action. Integration is the central innovation of the model, ensuring that all PHC functions work together to enable faster, safer and more effective responses to emergencies. This integration operates at three interconnected levels:

- **Workflow integration** – frontline processes are designed to serve multiple capacities simultaneously, reflecting how PHC functions in practice.
- **System integration** – shared enablers such as workforce development, supply chains, information systems and infrastructure support more than one capacity at the same time.
- **Outcome integration** – speed, safety and surge performance emerge from the coordinated functioning of **connect, detect, protect and treat** rather than isolated interventions.

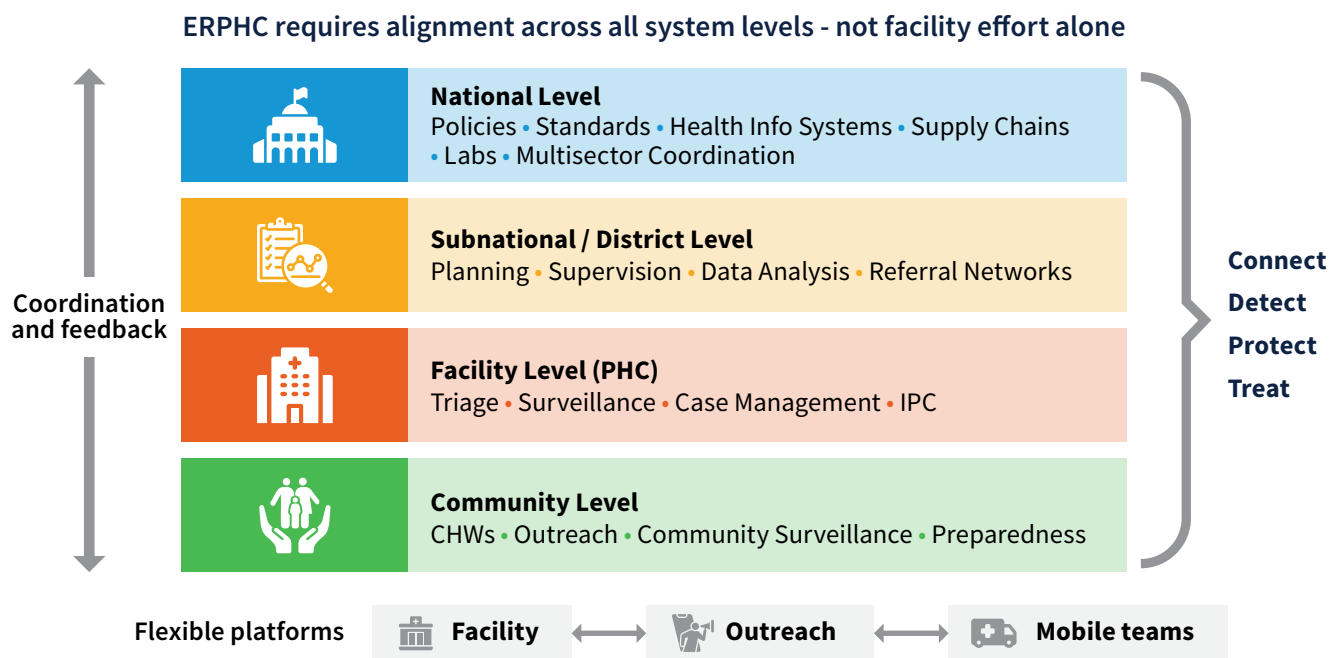
Implementation is tailored to each context and begins with a country assessment, planning and prioritization exercise, followed by co-creation of a national ERPHC strategy, customized to enable a resource-efficient, sustainable and impactful rollout. Further information on how ERPHC implementation is coordinated and integrated of is provided in the framework document that accompanies this operational guidance.

2.2 ERPHC implementation across health system levels

ERPHC is implemented as a coordinated, whole-system approach across national, subnational, facility and community levels of the health system (see Fig. 1).

Rather than focusing solely on primary health care facilities, ERPHC aligns policy, planning, resources and service delivery so that results at each level reinforce one another and collectively strengthen emergency readiness, response and recovery (see Table 1).

Fig. 1. Implementation levels of ERPHC



CHW: community health worker, IPC: infection prevention and control; PHC: primary health care

Table 1. Key focus for each level of ERPHC implementation

Level of implementation	Primary purpose in ERPHC	Key focal areas for implementation
National	Create the enabling environment for emergency ready PHC	Development of policies, standards and operational guidance; strengthening health information and early warning systems; national supply chain and logistics coordination; laboratory and referral network stewardship; multisectoral coordination and data-sharing arrangements, including one health linkages.
Subnational / district	Translate national plans into subnational and district operational readiness and oversight	District preparedness and response planning; routine analysis and use of surveillance and service data; facility supervision and mentorship; coordination with local authorities and partners; management of referral networks; ensuring availability and distribution of vaccines, diagnostics and medical countermeasures.
Facility	Deliver integrated PHC and emergency functions at the point of care	Screening and triage; event-based and routine surveillance and reporting; infection prevention and control; case management and referral; continuity of essential services planning; flexible service delivery arrangements during surges or disruptions.
Community	Strengthen early detection, trust and local response capacity	Community engagement and outreach; stakeholder mapping and mobilization; community feedback and reporting mechanisms; training and support of community health workers and community responders; risk communication and preparedness actions for households and vulnerable groups.

ERPHC: emergency ready primary health care; PHC: primary health care

2.3 Roles and responsibilities for implementing ERPHC (governance and ownership)

Implementing ERPHC requires clear leadership, well-defined roles and accountable governance across all levels of the health system. Although the model operationalizes emergency readiness at PHC and community levels, its success depends on coordinated decision-making, resource allocation and oversight from national to district levels. In centralized systems, national authorities focus on policy, standards and resource allocation, while subnational levels are primarily responsible for implementation. In decentralized systems, subnational authorities assume greater responsibility for planning, budgeting and operational within nationally defined frameworks. Across all contexts, effective ERPHC implementation requires clear national standards, robust two-way information flows and empowered frontline PHC and community actors with clearly defined authority and accountability. Further details on roles and responsibilities for implementing ERPHC are provided in *Annex 1*.

2.4 Financing and sustainability of ERPHC implementation

Adequate and predictable financing is critical to operationalizing ERPHC and requires sustained domestic investment in PHC as the foundation for prevention, preparedness, response and continuity of essential services. Budgets should be sufficient, equitably allocated and prioritize community-level delivery, integrated public health functions and multisectoral action. This includes embedding PHC and emergency readiness in national financing strategies; creating flexible, ring-fenced funds for surveillance, infection prevention and control (IPC), risk communication and community engagement (RCCE), medical countermeasures and service continuity and ensuring timely disbursement to subnational and frontline levels. Strategic financing reforms—such as rebalancing spending towards PHC, expanding fiscal space and using blended capitation¹ or performance-based payments—can incentivize integrated ERPHC functions and reduce financial barriers. In low-income or crisis-affected settings, health equity funds, community insurance and pooled humanitarian financing remain vital but should align with national priorities and progressively transition to harmonized, country-led systems in line with UHC2030 principles (10).

Sustaining ERPHC requires integrating preparedness and response into routine PHC governance, financing, workforce and service delivery, rather than treating them as temporary, externally funded activities. This integration depends on clear PHC mandates for preparedness and response, strong multisectoral and community leadership, continuity of core public health functions, a fit-for-purpose workforce including supported community health workers, resilient PHC infrastructure and supply chains and institutionalized learning systems such as simulations and after-action reviews. Embedding ERPHC within national health security, UHC and development strategies anchors emergency readiness in long-term policy frameworks and strengthens equity, resilience and community empowerment. *Annex 2* provides further details on financing and sustainability of ERPHC.

2.5 Cross-cutting enablers of ERPHC implementation

Effective implementation of ERPHC depends on system-wide enablers that underpin performance across all four core capacities and the three system outputs: **speed, safety and surge**. These enablers are not discrete programme components, but foundational system conditions that determine whether emergency readiness can function reliably in practice.

Key cross-cutting enablers include:

- strong leadership and governance to coordinate stakeholders, align policies and clarify accountability across all levels of the health system;
- a trained, supported (including through access to mental health and psychosocial support services) and adaptable health workforce with clearly defined emergency roles and supervision mechanisms;
- sustainable financing and procurement systems that ensure timely access to essential medicines, vaccines, diagnostics and supplies;

1 Blended capitation is a provider payment approach in which a health facility or primary care provider receives a fixed per-person payment (capitation) for a defined population, combined with other payment components (the “blend”), most commonly performance-based or fee-based elements.

- interoperable information, surveillance and communication systems that support decision-making and maintain community trust;
- resilient referral, logistics and infrastructure systems capable of functioning during periods of disruption; and
- interoperable digital health and data integration systems that enable timely threat detection, real-time monitoring of service continuity, multisectoral coordination and community engagement, thereby strengthening core capacities and improving the ERPHC outputs (see *Annex 3* for more details on digital health and data integration).

Strengthening these foundations ensures that emergency readiness is integrated into the broader PHC system rather than treated as a stand-alone function, enabling PHC to coordinate activities across **connect, detect, protect and treat** before, during and after emergencies.

Learning bite



System bottlenecks limit gains at facility level

While the ERPHC pilot demonstrated rapid improvements in facility-level readiness, performance remained constrained where system-level enablers were weak. Inconsistent availability of PPE and essential supplies, inadequate infrastructure for screening, isolation and waste, sanitation and hygiene (WASH) as well as workforce shortages and high staff turnover, all limited functional performance.

These findings reinforce that strengthening individual facilities is insufficient without concurrent investment in reliable supply systems, resilient infrastructure and a supported, stable health workforce. Without these system-level enablers, improvements in facility readiness are unlikely to translate into sustained performance at scale.

3. Initiating ERPHC at country level

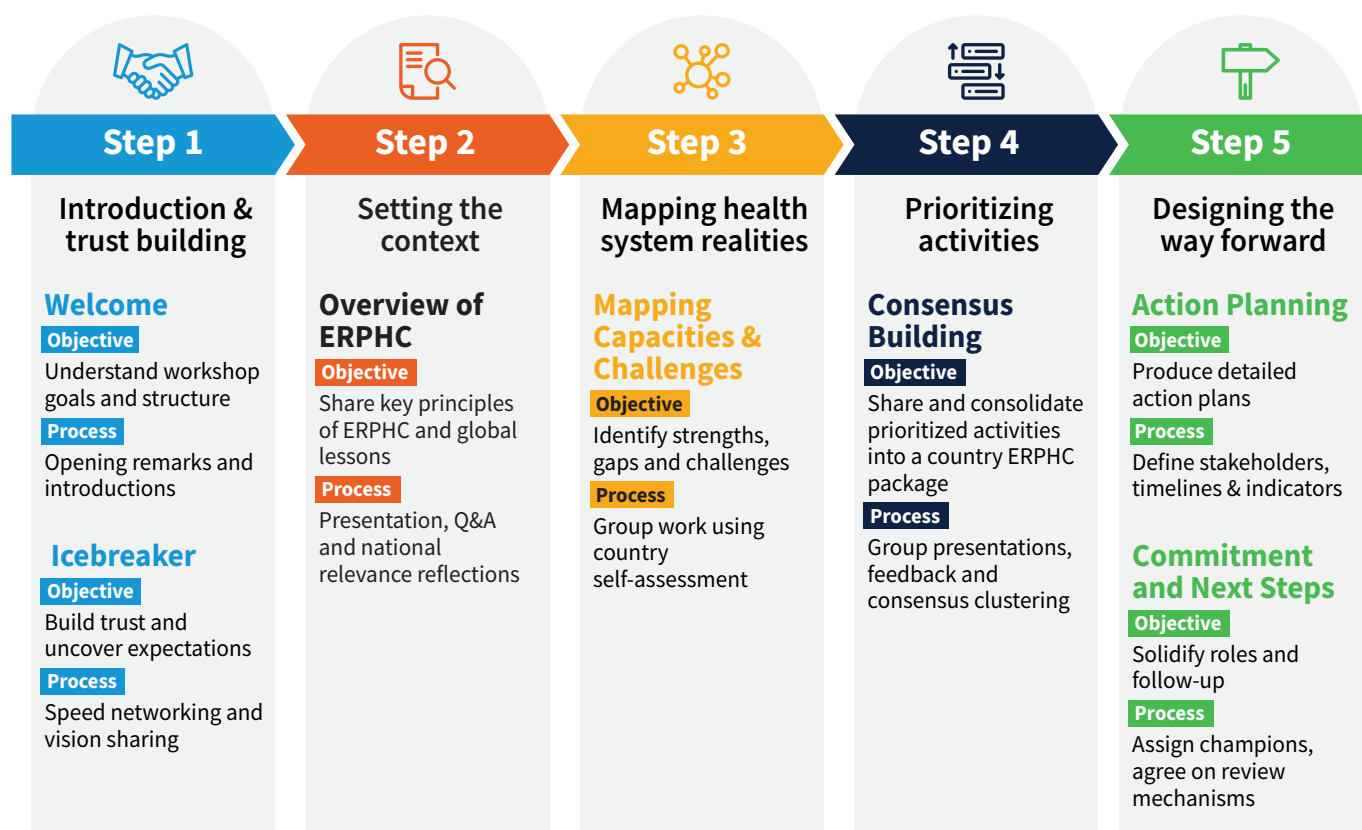
Country-level strategic and pre-implementation planning is essential for the successful implementation and sustainability of ERPHC. The purpose of pre-implementation planning is to secure buy-in and support for ERPHC across different levels of the health system and collaboratively develop a well contextualized ERPHC strategy, moving from the ERPHC Framework to a tailored country strategy. ERPHC pre-implementation planning involves initial engagement to ensure interest and alignment and consists of eight core steps as detailed below. As illustrated in Fig. 2, this process is iterative, with ongoing refinement as design and strategy are developed.

Effective implementation and institutionalization of ERPHC requires more than technical tools and infrastructure; it depends on sustained behavioural change and the integration of preparedness and response into routine PHC practice. Because ERPHC represents a complex system transformation involving multiple actors, priorities and institutional cultures, **deliberate change management is essential** to build shared ownership, align leadership and stakeholders, address resistance and support new ways of working. *Annex 4* consolidates the change management actions outlined across the operational guide into a practical approach to support leadership alignment, stakeholder engagement, communication, capacity building and continuous learning and to strengthen implementation quality and the long-term institutionalization of ERPHC.

3.1 Preliminary high-level engagement and alignment

This step focuses on initiating dialogue with key high-level decision-makers at the country level to assess their interest, priorities and potential support for the ERPHC initiative before proceeding with stakeholder analysis and mapping. It lays the foundation for political commitment and strategic alignment with country priorities, while helping to secure buy-in from influential actors early in the process.

Fig. 2. Steps to support country level ERPHC design



3.2 Stakeholder analysis and mapping

ERPHC intersects multiple technical sectors across ministries of health, ministries of disaster management and other relevant ministries at central and local levels, as well as national public health agencies, disaster management agencies and health programme implementing agencies (local, regional and international). The purpose of the stakeholder analysis is to identify core ERPHC stakeholders ensuring representation from community members, community health workforce, primary healthcare, emergency preparedness and response coordination, surveillance, IPC and clinical services. Once identified, stakeholders will be mapped to assess relationships between them, as well as their level of interest in and influence over ERPHC. The stakeholder analysis will also clarify the needs, concerns and expectations of the identified stakeholders.

The results of the stakeholder analysis and mapping will guide stakeholder consultations and help identify potential participants for the co-creation workshop. Stakeholder mapping should cover a wider range of actors than government agencies alone, including civil society organizations, community structures, community-based organizations, private sector entities and marginalized groups (see Annex 5).

3.3 Stakeholder engagement

Effective engagement with individual stakeholders based on their roles, interest and levels of influence, will be guided by a stakeholder engagement strategy. Initial engagement to generate interest in and support for ERPHC will depend on the specific country and context. High-level engagement at the minister or director-general level may be required before wider stakeholder engagement. It may be helpful to identify a lead department or division to host the initiative and high-level oversight of the ERPHC steering committee to ensure an integrated approach.

The initial engagement will introduce ERPHC as a concept, outline strategic approaches and present examples of intervention packages, while also providing an opportunity to gather initial insights and recommendations on potential contextualization. It may also be useful for ministries of health and other government stakeholders to discuss whether and which additional partners should be engaged. This can be done either in a short workshop format or at individual meetings.

A stakeholder engagement package of materials including a slide deck, one-pager explainers etc. will be useful for these meetings and are provided as part of implementation support.

3.4 Country assessment, planning, prioritization and consultations

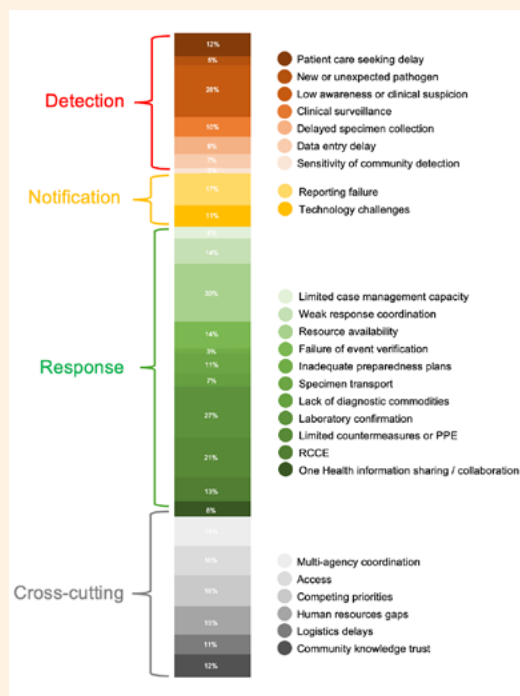
Country-level assessment for ERPHC begins with a rapid analysis of national hazards, vulnerabilities and PHC system readiness. This scoping phase enables countries to understand priority risks, existing capacities and key performance gaps before undertaking the structured ERPHC planning and prioritization process. Hazard and risk analysis may draw on existing national and global assessments, including Joint External Evaluation (JEE) reports (11), State Party Self-Assessment Annual Reporting (SPAR) submissions (12), 7-1-7 bottleneck analyses (13, 14), national risk registers and tools such as the WHO Strategic Toolkit for Assessing Risks (STAR) (9). Reviewing these sources enables countries to identify priority threats, contextual vulnerabilities and system constraints that shape PHC-level emergency readiness. Particular attention should be given to delays in detecting outbreaks and risks arising from other hazards, coordination gaps, workforce limitations, supply chain weaknesses and infrastructure constraints that affect speed, safety and surge performance. Findings from this initial review provide the contextual foundation for the ERPHC country self-assessment.

The purpose of the ERPHC country planning and prioritization process is to collect relevant information to contextualize the ERPHC approach and prioritize the intervention package. This process is not intended to be repeated periodically; it is designed as a one-time exercise to help a country determine which interventions to prioritize in developing an ERPHC strategy. The prioritization tool is structured around the HEPR framework (15), International Health Regulations (2005) health emergency core capacities (16), health system building blocks (17) and PHC operational priorities (6) to ensure alignment with global health security goals.

Learning bite



Using 7-1-7 bottlenecks to target ERPHC investments



Global analyses of 7-1-7 performance have identified recurring bottlenecks across outbreak detection, notification and response (13). Common reasons for delay in outbreak detection include low clinical suspicion and delayed care seeking. Notification bottlenecks frequently involve reporting failures and data flow gaps. Delays in responding to outbreaks are often linked to weak coordination, limited case management capacity, shortages of diagnostics and countermeasures and inadequate preparedness planning. Many of these bottlenecks originate at, or directly affect, the primary health care level, where outbreaks are first identified and initial response actions begin. As such, 7-1-7 bottleneck analysis provides a practical entry point for countries to identify PHC-level gaps that ERPHC can address.

The ERPHC pilot was intentionally designed to target one of the most common global detection bottlenecks: low clinical suspicion at frontline facilities. Pilot results demonstrated measurable gains. During the pilot, 214 confirmed cases of diseases with outbreak potential were detected – including mpox, measles, diphtheria, cholera, yellow fever and Lassa

fever – with 97% of cases detected within 7 days and 99% of cases reported within 1 day. During the mpox outbreak in Sierra Leone's, comparative analysis of 172 mpox cases across 12 ERPHC facilities and 10 non-ERPHC sites demonstrated statistically significant performance differences. ERPHC facilities detected cases faster, with a median detection time of 3 days compared to 5 days at non-ERPHC sites (P -value = 0.006), alongside improved safety practices and surge preparedness. Feedback from more than 1400 frontline health workers reinforced these findings: 99% reported improved clinical suspicion and strengthened epidemic readiness at their facilities.

These results illustrate how targeting 7-1-7 bottlenecks at PHC level can translate into measurable improvements in speed, safety and surge, by operationalising 7-1-7 at the frontline.

The country planning and prioritization process provides a structured overview of PHC-level emergency readiness across the four core capacities: **connect, detect, protect and treat**. It is not intended as a compliance exercise, but as a structured planning tool that enables countries to identify priority gaps, sequence actions and adapt technical options according to national risks, system capacity and operational realities. While the core capacities and system outputs – **speed, safety and surge** – remain constant, infrastructure requirements, workforce configurations and service delivery modalities will vary across PHC levels and geographic contexts. Activities may therefore be prioritized, adapted or deferred to ensure operational feasibility and alignment with national strategies.

The planning and prioritization process should be led by the country and coordinated through a designated ERPHC steering committee. The process should involve key informant consultations, document reviews and field visits with a multidisciplinary national team completing the ERPHC planning and prioritization tool.

Below is a proposed method for the country planning and prioritization process:

1. **Conduct a desk review to** analyse national policies and assessments such as Joint External Evaluation and States Parties Self-Assessment Annual Report (SPAR) annual reports, 7-1-7 data and health system

assessments for alignment with the HEPR framework (15), emergency capacities for the International Health Regulations (16) and PHC strengthening priorities (6).

2. **Review current indicators** related to the HEPR framework, International Health Regulations and PHC.
3. **Map existing programmes** (e.g. community-based surveillance, integrated disease surveillance and response (IDSR), and IPC to ERPHC's four capacities (**connect, detect, protect and treat**).
4. **Conduct key informant consultations** to collect required information for assessments through key informant interviews.
5. **Conduct field visits** to enable collection of data through observations and interviews with health workers.

Findings should be synthesized into a context-specific ERPHC roadmap that reflects national priorities, available resources and identified risks. *Annex 8* provides the country planning and prioritization tool, including key questions, data sources, scoring guidance and reflection prompts.

Learning bite



Using the self-assessment template - an example from a co-creation workshop in Nigeria

In Nigeria, the **ERPHC self-assessment was intentionally framed as a facilitated reflection and prioritization tool, rather than a formal performance measurement or compliance mechanism.**

During a co-creation workshop in Abuja, participants were organized according to the four core capacities – **connect, detect, protect and treat** – based on their roles and interests in the health system.

Each group used the structured self-assessment tool to collectively map current system strengths, gaps and implementation challenges. Rather than producing scores for external reporting, the self-assessment functioned as a practical instrument to support decision-making. Through facilitated discussions, each group identified systemic bottlenecks related to the list of activities within each technical package. This process generated a shared analytical baseline that enabled diverse stakeholders to develop a common understanding of where the health system was functioning well and where investments were most urgently needed.

Each group then presented its self-assessment findings to the others for feedback. After consolidating feedback, the findings were fed into structured prioritization exercises, including dot-voting, feasibility–impact matrices and world café discussions. In this way, the assessment results were translated directly into ranked intervention options, helping participants distinguish between “quick wins,” longer-term strategic investments and lower-priority actions. This iterative process ensured that prioritization was grounded in locally validated evidence rather than external benchmarks.

Importantly, the Nigerian experience demonstrated how the self-assessment can serve as a bridge between situational analysis and implementation planning. By embedding the assessment within a broader co-creation methodology, facilitators enabled participants to move seamlessly from identifying gaps to designing context-appropriate delivery models for various platforms of delivery. The process culminated in an agreed, prioritized package to be taken forward into resource mapping and implementation planning. Overall, the self-assessment process was explicitly designed to set the stage for prioritization and collective action, reinforcing its role as a tool for strategic alignment and operational planning rather than retrospective evaluation.

3.5 Co-creation of implementation strategy at country level

The initial co-creation workshop provides a structured forum for countries to consolidate findings from the assessment, planning and prioritization phase and translate them into strategic decisions. By convening relevant government actors and key stakeholders, the workshop focuses on adapting the ERPHC approach and prioritizing core interventions. Its objective is not merely discussion, but collective validation and endorsement of priorities to ensure political ownership, technical alignment and operational feasibility. Outcomes from the workshop should form the basis of a contextualized national ERPHC strategy and roadmap. Fig. 3 outlines the recommended steps for conducting an effective initial country co-creation workshop.

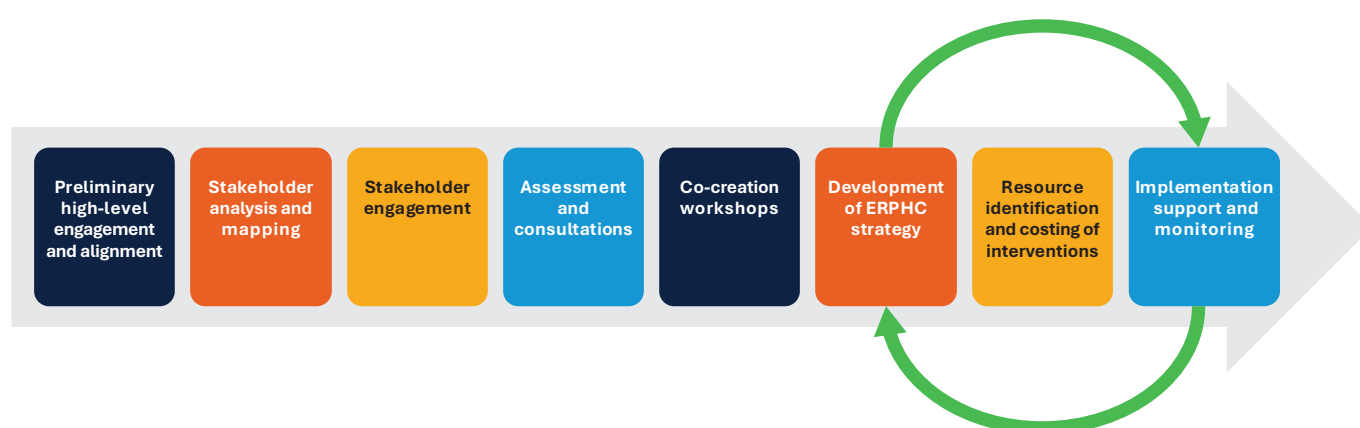
Subsequent co-creation workshops support operationalization. These sessions focus on pre-implementation planning, including finalising the monitoring framework, confirming assessment tools, aligning training materials, clarifying roles and responsibilities and sequencing rollout. Unlike the initial strategy workshop, they are typically smaller, decision-oriented meetings involving designated technical leads and key decision-makers to ensure timely agreement and implementation readiness.

3.6 Development of country ERPHC strategy

Outputs from the assessment, planning and prioritization process and co-creation workshops form the basis for developing a country-specific ERPHC strategy. This strategy should clearly articulate a contextualized ERPHC approach; a prioritized and sequenced intervention package across the four core capacities; an implementation plan; and a monitoring and evaluation framework aligned with national priorities.

While the assessment may identify priority activities within individual capacities – **connect, detect, protect and treat** – the strategy should avoid vertical or fragmented design. ERPHC emphasizes coordinated, outcome-oriented action. Activities should therefore be consolidated and integrated wherever possible so that investments strengthen multiple capacities simultaneously and enhance overall system performance, as outlined in the ERPHC framework (Section 4.2). The resulting ERPHC strategy should therefore present a unified roadmap, clearly demonstrating how prioritized actions across capacities are sequenced, integrated and aligned with national risks, health system realities and available resources.

Fig. 3. Steps for country co-creation workshop



Learning bite



Workflow-driven integration strengthens frontline performance

Results from the ERPHC pilot across the four countries demonstrated that strengthening activities that are integrated and aligned with workflows is central to improving epidemic readiness at the PHC level. Rather than delivering surveillance, IPC and case management as separate training streams, the intervention packages were deliberately structured around the real-world workflows of frontline health workers.

Capacity strengthening was integrated across diseases with epidemic potential and embedded within routine clinical practice. Health workers were equipped to detect and report suspected cases, protect themselves and patients and initiate immediate care within their existing responsibilities. This avoided fragmented, siloed training and reinforced competencies as interconnected functions rather than parallel tasks.

This practical integration was central to the pilot's success and indicates that workflow-driven, multi-competency strengthening provides a scalable foundation for strengthening PHC systems while operationalising epidemic readiness.

3.7 Resource identification and costing of interventions

The step of identifying resources and costing interventions should closely follow with the guidance provided on financing and sustainability of ERPHC (see *Section 2.5*). Resource identification for ERPHC is supported by a structured investment case template (see *Annex 6*) which helps countries systematically translate their ERPHC strategy into a practical financing and advocacy package, building on national assessment and co-creation processes. The template guides countries to clearly define priority results, identify and compile relevant evidence and data sources and present information in standardized tables to support costing, prioritization and decision-making. While adaptable to local contexts, the template ensures that essential financial and programmatic information is consistently captured to enable realistic planning, resource mobilization and coordinated scale-up of emergency ready primary health care.

3.8 Implementation planning, support and monitoring

Effective implementation and monitoring are essential for translating ERPHC from strategy into practice. This section describes how countries can plan, deliver and track emergency readiness across all levels of the health system, ensuring it is embedded within routine PHC functions and continuously improved through data-driven learning.

Implementation planning for ERPHC translates national priorities into sequenced, costed and context-specific actions at national, subnational, health-facility and community levels, ensuring that emergency readiness is embedded in routine PHC planning, budgeting and performance systems.

Implementation support should be tailored by level of the health system. At the **national level**, it focuses on policy alignment, standard setting, financing and integration within preparedness, PHC and health security frameworks. At the **subnational level** it strengthens planning, coordination, supervision and resource allocation and adapts national guidance to local risks. At the **health-facility level**, it supports operational readiness, workforce training, quality improvement, surveillance, IPC, referral pathways and continuity of essential services. At the **community level**, it strengthens RCCE, event reporting and the role of community health workers and civil society.

Monitoring ERPHC requires a systematic, multi-level approach aligned with national performance frameworks, PHC measurement systems and the health emergency and disaster risk management principles (18). Monitoring tracks progress in the four core capacities – **connect, detect, protect and treat** – across national, subnational, district, facility and community levels. Monitoring also assesses key structures, processes and outcomes for health system resilience, including essential public health functions, community engagement, surveillance timeliness, facility readiness, continuity of essential services and health worker safety.

ERPHC indicators should be integrated into existing health information systems, facility readiness assessments and surveillance platforms to avoid parallel reporting. These indicators should include both readiness indicators (such as IPC compliance, PPE availability and reporting performance) and functional indicators (such as 7-1-7 performance, simulation exercises and after-action reviews).

Data should flow bidirectionally among communities, facilities, districts, subnational and national levels, including through regular review sessions to support aggregation, trend analysis, feedback, corrective action, emergency planning and resource allocation. Routine monitoring should be complemented by periodic simulation exercises and by after-action reviews following real-life events to identify operational bottlenecks not visible through routine data. Findings should be reported through established national and subnational accountability mechanisms, including PHC performance reviews, health sector reviews and processes for IHR compliance and States Parties Self-Assessment Annual Reporting, to sustain prioritization of PHC emergency readiness and system resilience.

Learning bite



Structural versus functional readiness at PHC level

Monitoring during the ERPHC pilot revealed a structural–functional gap in readiness at the PHC level. Facilities that scored highly on structural readiness indicators — such as the availability of guidelines, tools, IPC materials and trained staff — did not consistently perform well during in situ simulation exercises, exposing operational weaknesses not captured by structural assessments. In particular, the simulations highlighted gaps in IPC implementation and in screening and triage workflows, including delays in case recognition, inconsistent PPE use, incomplete screening and breakdowns in reporting and referral pathways. These findings point to a persistent know–do gap between documented readiness and real-time performance under pressure. Preparedness initiatives must therefore be validated through real-world testing, rather than measured solely through structural compliance. Incorporating functional simulation alongside structural metrics provides a more accurate assessment of PHC-level readiness, making simulation, after-action reviews and iterative quality improvement essential to monitoring PHC emergency readiness.

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Annexes

Annex 1. Roles and responsibilities for operationalizing ERPHC

ERPHC operationalizes emergency preparedness, readiness, response and continuity of essential services where people receive care—both at PHC and community levels—while ensuring the governance, resources and coordination required at national, subnational and district levels. Table A1.1 outlines the roles and responsibilities of stakeholders across national, subnational, local/district and PHC/community levels. This information complements what is the ERPHC framework by clarifying the leadership, coordination and implementation functions needed to integrate primary care, essential public health functions, multisectoral action and community engagement into a unified, resilient and equitable emergency ready health system.

Table A1.1 Key roles and responsibilities for ERPHC implementation by level of health system

Level	Core role for ERPHC	Key roles and responsibilities
National level (ministry of health, national public health institute, National disaster management agency)	Establishes the enabling environment, national capacities and coherence for ERPHC, aligned with IHR (2005), HEPR and Health EDM.	Governance and policy: Set national direction for ERPHC by establishing strategies, standards, guidelines and legal and policy integration across health, PHC, emergency and multisectoral frameworks. National coordination and multisectoral action: Lead and coordinate national emergency operations, clarify PHC roles and institutionalize multisectoral collaboration with clear guidance for subnational and local implementation. Financing and resource mobilization: Secure and manage dedicated and flexible financing for ERPHC and ensure efficient, transparent procurement and supply systems aligned with PHC needs. Surveillance, information systems and laboratories: Strengthen integrated surveillance, laboratory networks and health information systems and ensure PHC is fully embedded in early detection, reporting, referral and feedback processes. Workforce development: Build and sustain a competent ERPHC workforce through national competency frameworks, training curricula and mentorship for PHC staff, CHWs, rapid response teams and supervisors. Supply chain and countermeasures: Ensure national readiness and rapid access to medical and public health countermeasures through buffer stocks, cold chain, WASH and logistics systems and rapid distribution mechanisms to PHC level.

Level	Core role for ERPHC	Key roles and responsibilities
Subnational level (regional/provincial health authorities)	Translates national standards into operational plans, allocates resources and provides technical and managerial oversight to districts and PHC facilities.	Regional planning and coordination: Translate national ERPHC strategy into region-specific action plans based on local and climate-related risks and convene multisector partners to coordinate implementation. Supervision and technical support: Strengthen district and facility performance through supportive supervision using ERPHC tools and deploy regional rapid response teams to support investigation, verification and surge response in coordination with PHC and community actors. Surveillance, laboratories and data use: Strengthen early warning and response through consolidation and analysis of district data, feedback to implementers and coordinated regional laboratory and sample transport systems with assured diagnostic capacity. Logistics and critical infrastructure: Ensure continuity of supplies, referrals and surge capacity by managing regional depots, cold chain backup and distribution systems and coordinating safe and scalable care resources.
Subnational (district/local level – district or sub-district health management teams and local government units)	Leads last-mile implementation through PHC facilities and community systems within defined catchments.	Local planning and micro-implementation: Develop and operationalize district or sub-district ERPHC micro-plans aligned with local preparedness plans, linking PHC and community systems and adapting national RCCE, IPC and surveillance guidance to local contexts. Local multisectoral coordination: Lead district-level coordination through incident management teams that bring together health, WASH, education, NGOs/CSOs, community leaders, CHWs and animal and environmental health actors. Surveillance, data use and public health action: Strengthen community-based surveillance by supervising PHC and CHW focal points, operationalising IBS/EBS triggers, verifying and reporting alerts and sharing surveillance data and community feedback with higher levels. Mentorship, quality improvement and simulation: Improve readiness and performance through routine supportive supervision, emergency simulations and after-action reviews covering IPC, RCCE, surveillance and case management. Local logistics and supply management: Maintain last-mile readiness by monitoring and redistributing supplies and coordinating transport for specimens, referrals and outreach services.

CHW: community health worker; CSO: civil society organization; EBS: event-based surveillance; ERPHC: emergency ready primary health care; EDRM: emergency and disaster risk management; HEPR: health emergency preparedness, response and resilience; IBS: indicator-based surveillance; IHR: international health regulations; NGO: nongovernmental organization; RCCE: risk communication and community engagement; WASH: water, sanitation and hygiene.

At the PHC facility level, roles and responsibilities align ERPHC core capacities – **connect, detect, protect and treat** – with health EDRM functions and the HEPR five Cs (collaborative surveillance, community protection, safe and scalable care, access to countermeasures and emergency coordination) (1,2). PHC facilities and community systems deliver frontline ERPHC functions as part of routine services, forming the foundation for early detection, rapid response, continuity of essential care and sustained community protection and trust. A mapping of roles and responsibilities by ERPHC capacity at the PHC and community levels is presented in Table A1.2; it applies to facility managers, PHC Teams, community health workers and community stakeholders.

Table A1.2 Mapping roles and responsibilities by ERPHC capacity at PHC facility and community level

ERPHC core capacity	Key roles and responsibilities at PHC and community system level
Connect – Community engagement and empowerment	Designate an RCCE focal person to work with CHWs, community leaders, CSOs and community health committees to co-produce risk communication, preparedness messages and social mobilization activities; operate community feedback channels (hotlines, in-person mechanisms, digital and social listening) and ensure feedback informs service improvement; and support community-based emergency risk assessments, household preparedness and participatory planning in line with WHO's community protection approach (1).
Detect – Surveillance and early warning	Assign a surveillance focal person to maintain IDSR (IBS/EBS) registers, detect priority events, report immediate alerts (preferably within 24 hours) and conduct event-based surveillance with community informants; maintain point-of-care diagnostics, specimen collection capacity and reliable referral pathways to district or regional laboratories; and strengthen health information systems for ERPHC functions, including linkage with community data and use of digital platforms (e.g. DHIS tools), according to country context and resources.
Protect – IPC, safe care, immunization and countermeasures	Establish and maintain IPC committees, triage and screening stations, isolation spaces, hand-hygiene systems, waste management, safe water and reliable energy in line with Health EDRM facility safety expectations; support and supervise context-specific IPC and WASH guidance, SOPs and tools, including community-level waste management; ensure availability of PPE, essential IPC supplies, functional cold chain and readiness for routine and supplemental immunization; and implement antimicrobial stewardship and prioritize health worker safety.
Treat – Case management, referral and essential service continuity	Apply national and district SOPs for triage, stabilization, case management (including infectious diseases, climate-related events and mass-casualty incidents) and referral, ensuring safe and timely transfers; maintain continuity of RMNCAH, NCD, mental health and chronic care services through catch-up systems, outreach and home-based care where appropriate; and conduct regular facility-level emergency simulations and participate in district drills.
Community systems (CHWs, community groups, civil society partners)	CHWs support health promotion, surveillance, early detection, follow-up of cases, home-based care, vaccination mobilization and continuity of care; community leaders and civil society organizations lead risk communication, social support, outreach to vulnerable groups and trust-building; and PHC facilities support and supervise CHWs and community partners in preparedness, readiness, response and community-based service continuity.

CHW: community health worker; CSO: civil society organization; DHIS: district health information system; EBS: events-based surveillance; EDRM: emergency and disaster risk management; IBS: indicator-based surveillance; IPC infection prevention and control; IDSR: integrated disease surveillance and response; NCD: noncommunicable disease; PPE: personal protective equipment; RCCE: risk communication and community engagement; RMNCAH: reproductive, maternal, neonatal, child and adolescent health; SOP: standard operating procedure; WASH: water, sanitation and hygiene.

Adaptation for centralized and decentralized systems

The roles and responsibilities framework applies to both centralized and decentralized governance models and clarifies decision-making authority and accountability at each level. In centralized systems, national authorities retain primary responsibility for decision-making and setting standards, while subnational and local levels focus on implementing and contextualising ERPHC. In decentralized systems, subnational authorities are accountable for planning, budgeting and activating emergency operations within national standards, while districts and local governments have greater autonomy to adapt interventions and engage communities. In both settings, effective ERPHC requires clear national standards, strong two-way information flows and empowered frontline PHC and community actors.

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2. Health emergency and disaster risk management framework. Geneva: World Health Organization; 2019 (<https://iris.who.int/handle/10665/326106>). Licence: CC BY-NC-SA 3.0 IGO.

All references accessed 15-17 April 2026

Annex 2. Financing and sustainability of ERPHC implementation

Adequate, predictable financing is essential to operationalising ERPHC, requiring sustained domestic investment in PHC as the foundation for prevention, preparedness, response and continuity of essential services. This includes embedding emergency readiness within national financing strategies, ensuring equitable allocation of resources to community-level service delivery and core public health functions, and establishing flexible funding for surveillance, IPC, RCCE, medical countermeasures and service continuity. Sustaining ERPHC also depends on integrating preparedness and response into routine PHC systems – through strong governance, a supported workforce, resilient infrastructure and supply chains, and continuous learning mechanisms – rather than relying on short-term, external funding. Aligning these efforts with national health security, UHC and broader development priorities strengthens long-term resilience, equity and country ownership. Table A2.1 summarises key financing changes needed to operationalise ERPHC, including prioritising PHC funding, increasing public investment and using strategic purchasing to support integrated services.

Table A2.1 Translating the key financing messages for operationalising ERPHC

Financing area	What needs to change	Purpose for operationalising ERPHC
Overall resource allocation	Increase the share of the health budget for PHC and reduce over-reliance on tertiary care.	• Improves system efficiency and resilience; and • avoids use of hospitals for primary care and emergency functions.
Public financing efforts	Increase health's share of government expenditure and increase PHC's share within the health budget.	• Creates sustainable fiscal space for PHC and emergency readiness.
Purchasing and payment mechanisms	Use strategic purchasing and blended capitation and performance-based payments.	• Incentivizes integrated ERPHC functions (surveillance, prevention, case detection, IPC and service continuity).
Financial protection and access	Design payment models that reduce out-of-pocket costs.	• Strengthens people-centred PHC and reduce financial barriers during emergencies.
External and partner financing	Use health equity funds, community insurance and humanitarian pooled financing in low-income and crisis settings.	• Sustains PHC delivery during and after emergencies.
Aid alignment and transition	Align partner financing with national priorities and progressively transition to country-led, harmonised systems (UHC2030 principles).	• Strengthens national ownership and long-term sustainability of ERPHC financing.

PHC: primary health care

Sustainability of ERPHC

Sustainability of ERPHC requires embedding emergency preparedness and response functions into routine PHC governance, financing, workforce and service delivery systems, rather than treating them as temporary, externally funded activities. As highlighted in WHO's guidance on PHC in health emergencies (1,2), PHC-oriented systems are inherently more resilient, able to prevent, absorb and recover from shocks while maintaining essential services.

Ensuring strong governance and stewardship would include clear mandates for PHC in preparedness and response, effective multisectoral coordination and community leadership and the ability to maintain core public health functions at PHC level such as surveillance, WASH, IPC and RCCE even when central systems are disrupted. A long-term workforce strategy is equally essential, centred on a fit-for-purpose PHC workforce, including community health workers, supported by continuous training, supervision and safe working conditions and by optimising skills mix, equitable distribution and integration of community-based services within PHC teams. Sustainability also

depends on resilient infrastructure, including PHC facilities with reliable WASH, power, connectivity, diagnostics and supply chains to enable safe care, surveillance, IPC and timely referral during protracted or climate-related crises. In addition, institutionalized quality improvement and learning systems—such as routine simulations, after-action reviews and community feedback—are needed to strengthen accountability, trust and adaptive performance. Finally, integrating ERPHC within national health security, UHC and multisectoral development strategies anchors emergency readiness in long-term policy frameworks and supports equity, resilience and community empowerment.

References¹

1. Primary health care and health emergencies. Geneva: World Health Organization; 2018 (<https://iris.who.int/handle/10665/328105>). Licence: CC BY-NC-SA 3.0 IGO.
2. World Health Organization, United Nations Children's Fund. Operational framework for primary health care: transforming vision into action. Geneva: World Health Organization; 2020 (<https://iris.who.int/handle/10665/337641>). Licence: CC BY-NC-SA 3.0 IGO.

All references accessed 15-17 April 2026

Annex 3. Digital health and data integration for ERPHC operationalization

Strengthening **digital health and data integration** is essential to the effective operationalization of ERPHC. Digital technologies and integrated information systems enable **timely detection of health threats, real-time monitoring of service continuity** and **rapid dissemination of actionable information** across all levels of the health system. The ERPHC Framework emphasizes that PHC facilities and community health workers require access to interoperable digital platforms to support surveillance, RCCE, countermeasure management and continuity of essential services. Digital integration enhances the performance of the **connect, detect, protect and treat** capacities by linking community-based information sources, PHC service data, public health surveillance and emergency coordination systems into a cohesive ecosystem.

Digital tools – including electronic medical records, community-based reporting apps, electronic IDSR systems, telemedicine and digital stock management systems—help overcome bottlenecks in early warning, health worker protection, continuity of care and supply chain management. The PHC Operational Framework identifies digital health as a key lever for system transformation and highlights its role in strengthening PHC governance, improving data quality, supporting clinical decision-making and enabling efficient resource allocation. In the context of emergency preparedness and readiness, digitalization also supports rapid verification of alerts, automated triage routing, digital supervision and remote support to frontline health workers.

Integrated digital systems also facilitate **multisectoral coordination**, aligning health data with meteorological, environmental, laboratory and emergency management systems, consistent with Health EDRM's emphasis on cross-sector information sharing and early warning. Ensuring interoperability between PHC information systems, national surveillance platforms, laboratory information systems and EOC (emergency operations centres) reduces delays and fragmentation. Leveraging mobile technologies and community-level digital tools strengthens community engagement and empowers CHWs and civil society actors to report events, disseminate risk information and support vulnerable populations during crises.

To support implementation, countries should prioritize:

1. **Interoperable, standardized digital architectures** that integrate PHC facility data, community health data, surveillance signals and laboratory data;
2. **Real-time dashboards and analytics** for monitoring 7-1-7 performance, service continuity, IPC and stock levels.
3. **Digital capacity-building** for PHC providers and CHWs, including digital literacy, data use and ethical data governance;
4. **Investments in connectivity and device availability** at PHC and community levels; and
5. **Data governance frameworks** that protect privacy, ensure ethical use and promote transparency.

By embedding digital and data integration into ERPHC implementation, countries can enhance the **speed, accuracy and equity** of their emergency readiness efforts and build resilient PHC systems capable of responding effectively to all-hazards threats.

Annex 4. Change management framework for ERPHC Implementation

Introduction

The successful implementation and institutionalization of ERPHC requires more than the introduction of technical tools, guidelines and infrastructure. It depends fundamentally on the ability of health systems, institutions and individuals to adopt new ways of working, sustain behavioural change and embed emergency preparedness and response functions into routine primary health care practice. Without deliberate and systematic change management, even well-designed interventions risk limited uptake, fragmented implementation and poor sustainability.

Health systems operate within complex political, institutional and social environments, characterized by multiple stakeholders, competing priorities, resource constraints and entrenched organizational cultures. Introducing ERPHC therefore represents a significant system-level transformation, requiring shifts in leadership practices, governance arrangements, frontline behaviours, intersectoral coordination and community engagement. Effective change management is essential to navigate this complexity, build shared ownership, mitigate resistance and align actors around a common vision for emergency readiness and resilience.

This annex consolidates and synthesizes the change management processes required throughout *sections 3.1–3.8* of the operational guide. By presenting these elements in a coherent framework, it provides countries and partners with practical guidance to support leadership alignment, stakeholder mobilization, communication, capacity building and continuous learning throughout the ERPHC lifecycle. The annex is intended to strengthen implementation fidelity, enhance system readiness and support the long-term institutionalization of ERPHC within national and subnational health systems.

Change Pathway

The ERPHC change management pathway operates across five interrelated domains:

- 1. Leadership and alignment**
Early engagement of high-level decision-makers establishes political commitment, aligns ERPHC with national priorities and signals institutional support for change.
- 2. Governance and coalition building**
Formal steering committees and guiding coalitions provide shared accountability, coordination mechanisms and sustained advocacy for ERPHC implementation.
- 3. Communication and engagement**
Structured communication strategies and stakeholder engagement processes create shared understanding, reinforce urgency, manage expectations and promote collective ownership.
- 4. Empowerment and capability development**
Training, mentorship, delegation of authority and provision of resources enable frontline workers and managers to translate strategy into practice.
- 5. Learning, adaptation and institutionalization**
Continuous feedback, monitoring, recognition of early wins and integration into standard operating procedures support learning, adaptation and long-term sustainability.

Progress across these domains enables the transition from initial mobilization and planning to system-wide adoption and institutionalization of ERPHC.

Expected outcomes

Effective application of this change management framework is expected to result in:

- strong and sustained political and institutional commitment to ERPHC;
- coordinated, multi-sectoral implementation structures;
- high levels of stakeholder and community ownership;
- improved workforce motivation, competence and accountability;
- adaptive implementation informed by evidence and learning; and
- integration of ERPHC principles into routine policy, financing and operational systems.

Collectively, these outcomes contribute to resilient, responsive and people-centred primary health care systems capable of preventing, detecting and responding to health emergencies while maintaining essential services.

Change management examples

The following section provides a non-exhaustive list of examples of change management processes that may be necessary at different stages within ERPHC design.

Process area	Actions
Preliminary high-level engagement and alignment (see Section 3.1)	• explicitly communicate the urgency of emergency preparedness by sharing data, case studies and lessons from recent outbreaks and disasters; and • incorporate messaging on the urgency of preparedness into all stakeholder engagement activities and materials.
Stakeholder analysis and mapping (see Section 3.2)	• formalize a country core ERPHC steering committee or guiding coalition, including influential leaders from government, civil society, community organizations and the private sector; • ensure this coalition is empowered, visible and tasked with championing ERPHC at all levels; and • define clear roles, responsibilities and mechanisms for coordination, collaboration and decision-making within this group.
Stakeholder engagement (see Section 3.3)	• develop a comprehensive communication plan to cascade the ERPHC vision and strategy across all levels of the health system and to communities; • explicitly communicate the urgency of emergency preparedness by sharing data, case studies and lessons from recent outbreaks and disasters; • use multiple channels (meetings, workshops, digital platforms, community forums) and adapt messaging to different audiences; • highlight the risks of inaction and the benefits of rapid ERPHC implementation. • incorporate messaging on the urgency of preparedness into all stakeholder engagement activities and materials; and • ensure regular updates and feedback loops so stakeholders see progress and remain engaged.

Process area	Actions
Country assessment and consultations (see Section 3.4)	• secure visible endorsement of the country assessment process from Ministry of Health leadership to signal legitimacy and encourage participation across sectors; • frame the assessment as a collaborative learning exercise rather than an evaluation, reducing fear of judgement and encouraging honest reflection; • establish a multidisciplinary country assessment team representing national, subnational and frontline levels to ensure diverse perspectives and shared ownership of findings; • use key informant and inclusive consultations (community representatives, frontline health workers, etc) to build trust, identify perceived barriers to implementation and surface informal system dynamics that may not appear in written documents; • facilitate structured reflection sessions with stakeholders to review preliminary findings, confirm priorities and build consensus on readiness gaps; • communicate assessment findings transparently and share results widely to support collective understanding of system strengths, challenges and opportunities; and • use assessment results to create a shared sense of urgency and readiness for change, linking findings directly to subsequent co-creation and strategy development phases.
Co-creation workshops (see Section 3.5)	• collaboratively develop a clear, compelling vision statement for ERPHC with input from the guiding coalition and broader stakeholders; • ensure the vision addresses both the technical and human aspects of change (e.g. improved health worker safety, community trust); • translate the vision into a strategic roadmap with measurable objectives and milestones and communicate this widely; • identify and remove barriers (e.g. bureaucratic, resource, knowledge) that could impede ERPHC implementation; • delegate authority to local champions and teams to adapt and act on the ERPHC strategy within their context.
Resource identification and costing of interventions (see Section 3.6)	• early engagement of finance and planning authorities (e.g. ministries of finance, budgeting units and donor coordination platforms) to build shared understanding of ERPHC priorities and resource implications; • establish a cross-functional costing and financing working group that includes technical experts, planners, finance officers and implementation leads to ensure realistic and jointly owned costing assumptions; • use participatory costing workshops to align stakeholders on priority interventions, clarify assumptions and reduce resistance to resource allocation decisions; • ensure transparent discussion of trade-offs and prioritization decisions to build trust and reduce perceptions of competition between programmes; and • strengthen local ownership by involving subnational managers and frontline implementers in estimating resource needs and feasibility.
Development of ERPHC strategy (see Section 3.7)	• ensure regular updates and feedback loops so stakeholders see progress and remain engaged; • identify and remove barriers (e.g. bureaucratic, resource, knowledge) that could impede ERPHC implementation; • delegate authority to local champions and teams to adapt and act on the ERPHC strategy within their context; and • set clear, achievable short-term goals (e.g. training completion, first facility readiness assessments, improved reporting rates).

Process area	Actions
Implementation support and monitoring (see <i>Section 3.8</i>)	• provide resources, training and support to empower frontline workers and facility managers to implement changes; • after initial successes, review lessons learned and adjust strategies to address new challenges and opportunities; • expand the change effort by scaling up successful interventions, integrating feedback and involving new stakeholders; • publicly recognize and celebrate early achievements at all levels (facility, district, national); • promote stories of success and testimonials from frontline workers and community members; • use short-term wins to build momentum, increase stakeholder confidence and attract further support; • use data and stories from early wins to advocate for additional resources and policy support; • ensure ongoing leadership commitment and institutionalize ERPHC as a core part of the health system's identity and routines; and • work with the ERPHC Steering Committee to embed ERPHC principles into standard operating procedures, training curricula and performance evaluations.

Annex 5. Stakeholder analysis and mapping

The purpose of stakeholder analysis and mapping is to identify key players for ERPHC implementation, assess their levels of interest and influence, their needs, concerns, expectations and plan engagement strategies. The following process is undertaken:

1. Identify stakeholders

The process begins with desktop review of stakeholders in the country of interest that are involved in community members, community health, primary healthcare, emergency preparedness/response, surveillance, IPC and clinical services. The stakeholders can be derived from a few categories:

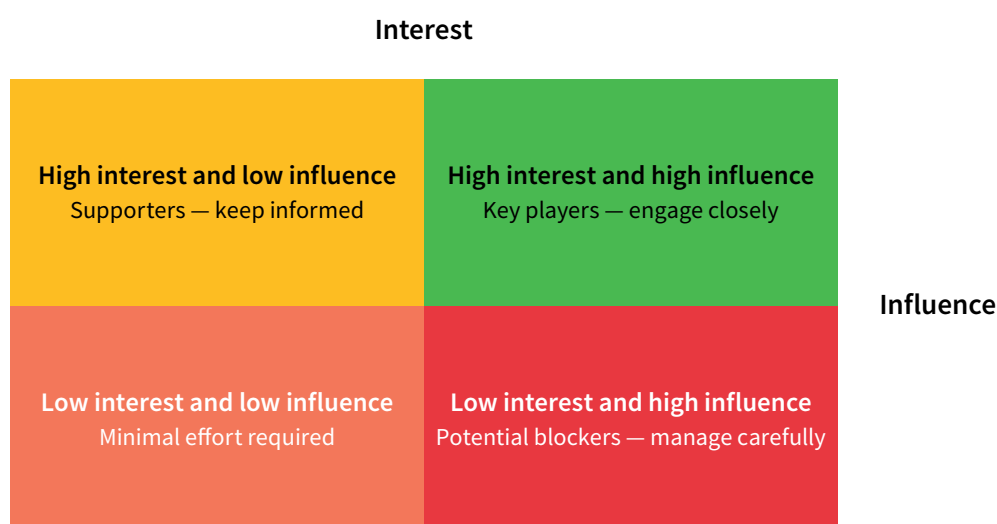
Stakeholder category	Examples
Government entities	ministries of health (central and local), finance, disaster management/emergency services, national public health institutes
Healthcare providers	PHC facilities, CHWs, private clinics, traditional medicine practitioners
Community representatives	local leaders, faith-based organizations
Regional and international partners	World Health Organization, United Nations Children's Fund, Africa Centres for Disease Control and Prevention, international nongovernmental organizations
Civil society organizations	civil society organizations and local nongovernmental organizations
Private sector	pharmaceutical companies, logistics providers
Academia and research	universities, think tanks, public health institutes

2. Map stakeholders

The stakeholders can be mapped according to the following parameters:

- category – see list above;
- interest – level of interest in ERPHC (e.g. high/medium/low);
- influence – capacity to affect the project positively or negatively (e.g. high/medium/low);
- role/contribution – their role as it relates to ERPHC and how they may contribute; and
- engagement strategy – best way to engage using the interest/influence matrix (see Fig. A5.1 below).

Fig. A5.1. Influence/interest matrix and related engagement strategies



The information gathered in the desktop mapping is then populated into a table (see example in Table A5.1). Details of the primary contact person and their contact information, as well as links to relevant websites or materials should also be captured. Once a preliminary mapping has been done, further brainstorming sessions and consultations with local experts are done to refine and expand on the mapping.

Table A5.1 Example of a preliminary stakeholder mapping

Stakeholder	Category	Interest	Influence	Role/ contribution	Engagement strategy
Ministry of health (separate for national and subnational levels)	government	high	high	policy development and coordination	regular meetings and updates
National public health Institutes	government	high	high	policy development and coordination	regular meetings and updates
Local nongovernmental organizations	community	medium	medium	community mobilization	workshops and consultations
WHO	international partner	high	high	technical support and funding	strategic partnership meetings
UNICEF	international partner	high	high	technical support and funding	strategic partnership meetings
Private clinics	healthcare provider	medium	low	service delivery at phc level	information sharing
Community leaders	community	medium	high	community mobilization	information sharing, consultations
PHC health workers	service provider	high	medium	service provision	workshops, regular updates
CHWs	community	high	medium	community mobilization	information sharing, consultations

CHW: community health worker.

Annex 6. Template for developing a country-specific ERPHC investment case

This annex provides a template to support development of the ERPHC investment case, focusing on key inputs, data sources and decision-making needs. The template should be completed through a collaborative, country-led process involving ministries of health, finance, and key stakeholders (e.g. subnational authorities, partners and implementing agencies), building on findings from ERPHC assessments and co-creation workshops. It is expected that countries will adapt the template to their context—refining sections, data inputs and tables as needed—while ensuring that core information required for prioritisation, costing and financing decisions is captured.

Section 1. Executive summary

The aim of this section is to allow readers to quickly understand why ERPHC investment is needed, what it will achieve and what decisions are required.

Element	Summary
Country	
Lead ministry	
Timeframe	
Total investment required	
Key outcomes	
Main financing sources	
Funding gap	

Section 2. Strategic context and rationale

This section explains why ERPHC investment is necessary within the country context. It links ERPHC to national priorities, identifies system vulnerabilities and demonstrates alignment with existing health security commitments.

2.1 Country context and burden of disease

This section provides concise guidance to support development of the ERPHC investment case, focusing on key inputs, data sources and decision-making needs.

Typical sources of information: National health sector reports; Ministry of Health epidemiological profiles; WHO country profiles; Demographic and Health Surveys; routine health management information system and DHIS2 data; estimates from Institute of Health Metrics Evaluation; surveillance reports; outbreak reports; national Sustainable Development Goals reports.

2.2 Recent shocks and impacts

This subsection captures recent emergencies, outbreaks or disasters that have exposed weaknesses in the health system. Countries should briefly describe the health, service delivery and socio-economic impacts of these events. The purpose is to provide practical context and illustrate the consequences of inadequate preparedness.

Typical sources of information: Outbreak investigation reports; disaster response reviews; after-action reviews; national emergency reports; WHO situation reports; ministry of health emergency dashboards; economic impact analyses.

Event	Year	Health impact	Economic/social impact

2.3 National policy alignment

This subsection describes how ERPHC investments support existing national and international policy commitments. It should list key strategies, frameworks, or plans and briefly explain how ERPHC contributes to their priorities. The intention is to demonstrate alignment rather than duplication, showing that ERPHC strengthens ongoing health system and emergency preparedness agendas.

Typical sources of information: National health strategic plans; UHC strategies; disaster risk management frameworks; National Action Plans for Health Security; IHR/JEE/SPAR reports; PHC policies; partner programme frameworks.

Framework / Strategy	Relevant priority	Link to ERPHC
National health plan		
UHC strategy		
Disaster risk plan		
IHR / JEE / SPAR		
Other relevant policies		

2.4 System Vulnerabilities

This section provides concise guidance to support development of the ERPHC investment case, focusing on key inputs, data sources and decision-making needs.

Typical sources of information: Country self-assessment results; JEE/SPAR findings; 7-1-7 analysis; IPC assessments; service readiness assessments (e.g. service availability and readiness assessment and harmonized health facility assessment); stakeholder consultation reports; supervision reports.

Capacity	Key gaps
Connect	
Detect	
Protect	
Treat	

Section 3. Priority Investment Areas

This section translates assessment findings into specific investment priorities. It defines what will be funded and ensures investments are linked to country needs and readiness levels.

3.1 Intervention Package

This subsection describes the specific ERPHC interventions that will be funded. It should include interventions agreed during prioritization and co-creation processes and describe where they will be implemented, who will lead implementation and the expected timelines. The emphasis should be on translating strategy into concrete investment actions.

Typical sources of information: Co-creation workshop outputs; prioritization exercises; ERPHC maturity assessment; national technical guidelines; stakeholder consultation notes; implementation planning documents.

Note: If the ERPHC package was consolidated around cross-cutting themes (e.g. workforce, infrastructure, governance, information systems and community engagement), then the intervention package described here should be organized according to those themes instead of according to each capacity.

Example – Connect (repeat for detect, protect and treat capacities)

Intervention	Target area	Coverage	Lead agency	Timeline
RCCE training				
Digital platforms				

3.2 Geographic and population coverage

This subsection defines the scope of implementation across national, subnational and facility levels. It should describe which areas or populations will be prioritized and provide estimates of coverage where possible. Information included here should reflect realistic implementation capacity and phased expansion plans.

Typical sources of information: Population census data; district health profiles; facility mapping; service coverage data; GIS mapping; PHC facility registries.

Level	Target	Units	Population covered
National			
Provincial/Regional			
District			
Facility			

Section 4. Costing and resource requirements

This section uses costing to translate technical priorities into financial requirements. It allows governments and partners to understand affordability, plan budgets and identify gaps.

4.1 Summary budget

This subsection provides an aggregated financial overview of the investment package. It should present annual and total projected costs by major budget categories, ensuring figures align with intervention priorities defined earlier. The summary budget should provide a clear picture of overall resource requirements.

Typical sources of information: Programme budgets; national costing tools; partner budgets; procurement plans; previous project expenditure data; government financial planning documents.

Cost Category	Year 1	Year 2	Year 3	Total
Infrastructure				
Human resources and training				
Supplies				
Digital systems				
Measurement, evaluation and learning				
Management				
Total				

4.2 Detailed Costing

This subsection breaks down the costs of individual activities to increase transparency and support planning. It should provide details on unit costs, quantities, frequency of implementation and total annual costs for each activity. Costing assumptions should be clear and based on available local data where possible. This subsection is optional and can be an annex.

Typical sources of information: Procurement price lists; salary scales; training budgets; logistics costs; infrastructure costing estimates; previous programme budgets.

Activity	Unit cost	Quantity	Frequency	Annual cost

4.3 Sustainability costs

This subsection identifies costs that will continue beyond initial implementation or external funding periods. It should describe ongoing operational expenses such as supervision, maintenance, or supply replenishment and indicate which institutions will assume responsibility for long-term financing.

Typical sources of information: Medium-term expenditure frameworks; recurrent expenditure budgets; human resources plans; maintenance contracts; supply chain forecasts.

Cost type	Post-project annual cost	Responsible entity
Supervision		
Maintenance		
Supplies		

Section 5. Expected returns on investment

This section demonstrates value-for-money. Decision-makers are more likely to invest when health outcomes and economic benefits are clearly described.

5.1 Health returns

This subsection describes the health outcomes expected from ERPHC investment. Countries should select measurable indicators that reflect improvements in preparedness, response, or service continuity and include baseline values and targets where data are available. Indicators should be realistic and relevant to national priorities.

Typical sources of information: Baseline assessments; routine HMIS data; surveillance indicators; facility performance reports; 7-1-7 data.

Indicator	Baseline	Target	Timeframe
Detection time			
Notification time			
Response time			
Health worker infections			
Service continuity			

5.2 Economic and social returns

This subsection explains how ERPHC investment contributes beyond health outcomes, such as reducing emergency-related costs, protecting household livelihoods, or improving system efficiency. It should describe anticipated benefits and reference available evidence or reasonable assumptions supporting these expectations.

Typical sources of information: Economic analyses; outbreak costing studies; health financing analyses; labour/productivity reports; national economic impact studies.

Area	Expected benefit	Evidence/assumption
Outbreak costs		
Productivity		
Household protection		

5.3 Scenario analysis

This subsection presents comparative scenarios to demonstrate the value of investment. Countries may describe expected outcomes under different levels of investment, such as no investment, partial investment, or full implementation. This analysis may include qualitative information where robust modelling is not available or to augment modelled scenarios.

Typical sources of information: Previous emergency response cost data; modelling tools; expert consultations; historical programme outcomes.

Scenario	Health impact	Economic impact
No investment		
Partial investment		
Full ERPHC		

Section 6. Financing landscape and gaps

This section allows countries to maximize efficiency, avoid duplication and identify realistic funding gaps by reviewing existing financing flows.

6.1 Existing funding

This subsection maps current funding sources supporting relevant activities. Countries should identify available domestic and partner financing, indicate the duration of funding and describe which interventions are currently covered. This helps clarify what is already financed and where alignment exists.

Typical sources of information: National budgets; donor grant agreements; partner workplans; Ministry of Finance allocations; programme financial reports.

Source	Amount	Duration	Linked activities
Ministry of Health budget			
Local government budgets			
Donor grant agreements			

6.2 Financing gaps

This subsection compares investment needs with available resources to identify funding gaps. It should quantify gaps for each ERPHC capacity or major activity area. The purpose is to clearly identify areas requiring additional mobilization.

Typical sources of information: Costing results; budget analyses; partner financing maps; resource tracking exercises.

Cost Category	Required	Available	Gap
Infrastructure			
Human resources and training			
Supplies			
Digital systems			
Measurement evaluation and learning			
Management			
Total			

6.3 Potential new financing options

This subsection outlines possible financing mechanisms that could close identified gaps. It should describe feasible options such as government budget allocations, partner financing, contingency funds, or other innovative mechanisms and indicate expected timelines and responsible actors.

Typical sources of information: Health financing strategies; climate financing opportunities; contingency funds; development partner financing pipelines; private sector engagement strategies.

Mechanism	Feasibility and considerations	Timeline	Lead
Contingency Fund			
Climate Fund			
Blended Finance			
Private Sector			

Section 7. Implementation and phasing plan

This section articulates a phased approach that ensures investments follow system maturity and readiness, allowing foundational activities to be implemented first.

7.1 Phased roll-out

This subsection describes how implementation will be sequenced over time. It should explain phases such as pilot, scale-up and consolidation, including key objectives and outputs for each stage. Phasing should reflect system readiness and available resources.

Typical sources of information: ERPHC implementation strategy; pilot results; readiness assessments; programme roadmaps; stakeholder agreements.

Phase	Period	Focus	Key outputs
Phase 1		pilot	
Phase 2		scale	
Phase 3		consolidate	

7.2 Annual milestones

This subsection presents key implementation milestones to be achieved each year. Countries should identify measurable outputs or deliverables and indicate responsible entities. These milestones should support monitoring and accountability throughout implementation.

Typical sources of information: ERPHC implementation strategy, workplans; annual operational plans; programme monitoring frameworks.

Year	Milestone	Responsible
No investment		
Partial investment		
Full ERPHC investment		

7.3 Risk management

This subsection identifies potential risks that may affect implementation and describes measures to mitigate them. It should consider operational, financial, or governance risks and outline practical mitigation strategies to reduce their impact.

Typical sources of information: Risk registers; programme evaluations; partner risk analyses; lessons learned from previous emergencies.

Risk	Likelihood	Impact	Mitigation

Section 8. Governance and accountability

This section articulates governance and accountability mechanisms by describing roles, decision-making and oversight, to increase credibility and ensure transparent use of resources.

8.1 Oversight structure

This subsection describes the governance arrangements that will guide implementation. It should identify key coordination or oversight bodies, clarify their roles and describe how cross-sector collaboration will be supported.

Typical sources of information: Terms of reference; committee structures; governance frameworks; ministry organizational charts.

Body	Role	Membership
Ministry of health		
ERPHC steering committee		
Technical working group		

8.2 Financial management

This subsection explains how financial resources will be managed. It should describe budgeting, procurement and reporting arrangements, including which systems or institutions will be used to ensure transparency and accountability.

Typical sources of information: Public financial management guidelines; procurement procedures; financial reporting templates; audit frameworks.

Function	Responsible unit	System of management
Budgeting		
Procurement		
Reporting		

8.3 Performance monitoring

This subsection is linked to the section above and describes how progress will be tracked throughout implementation. Countries should identify key indicators, data sources and reporting frequency to support ongoing monitoring and adaptive management.

Typical sources of information: ERPHC implementation strategy; measurement evaluation and learning framework; indicator reference sheets; routine reporting systems; supervision tools.

Indicator	Source	Reporting	Frequency

Section 9. Advocacy and engagement strategy

This section ensures clear communication to stakeholders to secure funding and political support.

9.1 Target audiences

This subsection identifies the primary audiences needed to secure support for ERPHC investment. It should describe the objectives of engaging each stakeholder group and tailor messaging to reflect their interests and priorities.

Typical sources of information: Stakeholder mapping; political economy analysis; partner engagement plans.

Audience	Objective	Key message	Channel
Ministry of finance			
Cabinet			
Donors			
Parliament			

9.2 Communication products

This subsection outlines the materials required to support advocacy and engagement efforts. It should describe the types of communication products needed, their intended audiences and when they will be developed or used.

Typical sources of information: Existing policy briefs; communication strategies; dashboard tools; presentation materials.

Product	Purpose	Audience message	Timeline
Policy brief			
Slide deck			
Dashboard			

Annex 7: Approach to development of the ERPHC framework and operational guide

Background

The COVID-19 pandemic highlighted the critical need for epidemic ready primary health care that can detect outbreaks early, maintain essential services and protect communities and health workers through strong integration with public health systems (1). Strengthening PHC improves detection, vaccination, treatment and coordination, but requires stepwise progress, sustained financing and political commitment. Ultimately, integrated and resilient PHC systems are essential to prevent future pandemics and improve overall health outcomes.

The emergency ready primary health care (ERPHC) framework was conceptualised in response to lessons from COVID-19, which highlighted the critical role of primary health care (PHC) in health security. This accompanying operational guide was developed based on lessons learnt from ERPHC implementation and global best practice.

Process for developing the ERPHC Framework

The framework and operational guide were developed through an iterative process that combined analytical work with practical learning and continuous peer review and feedback.

Initial work at RTSL focused on defining the core capacities required at the PHC level to prevent, detect and respond to health threats, leading to the articulation of four interdependent functions: Connect, Detect, Protect and Treat. Early efforts to map activities directly from existing frameworks, such as the International Health Regulations (2005), proved insufficient, as these are largely oriented toward national-level capacities and did not translate easily to frontline implementation. The approach was therefore reframed, starting from the health facility and community level to define the practical capacities required for health security in routine PHC workflows. Activities were then derived based on what is needed to operationalise each capacity in practice. A comprehensive review of global frameworks – including International Health Regulation benchmarks, the Joint External Evaluation tool, the HEPR framework, the health emergency and disaster risk management framework and broader PHC systems strengthening guidance – was subsequently undertaken to map and align relevant activities ensuring coherence with global standards while translating them into an integrated, operational model for PHC-level preparedness and resilience.

Alongside this, a four-country ERPHC pilot was implemented to test a subset of activities, generating learning and evidence of their effectiveness in improving epidemic readiness. The pilot demonstrated substantial improvements in epidemic readiness, surveillance and infection prevention and control, alongside faster detection and reporting of diseases (2). These results highlight the effectiveness of integrated PHC systems and targeted mentorship in rapidly enhancing health security outcomes, despite ongoing challenges such as supply and infrastructure gaps. Several success stories on ERPHC implementation have been published through the RTSL website:

- <https://resolvetosavelives.org/strategies-in-action/how-fake-patients-are-helping-real-health-facilities-prevent-epidemics/>
- <https://resolvetosavelives.org/strategies-in-action/how-epidemic-ready-health-facilities-in-sierra-leone-stopped-measles-in-its-tracks/>
- <https://resolvetosavelives.org/strategies-in-action/epidemic-ready-health-care-workers-in-uganda-sound-the-alarm-to-end-outbreaks-faster/>
- <https://resolvetosavelives.org/strategies-in-action/connect-detect-protect-treat-learnings-from-implementing-epidemic-ready-primary-health-care/>

Technical review and validation process

The ERPHC framework and operational guide underwent a multi-phase, iterative review process combining document review, consultations and structured feedback synthesis:

Phase 1: WHO Technical Reviews (June–August 2025)

- Detailed, line-by-line feedback and review across all sections and activities was provided by seven technical staff from WHO Emergencies Programme. Each capacity (Connect, Detect, Protect, Treat) and its activities received input from multiple reviewers with feedback focused on technical accuracy, clarity and completeness.
- Five members of technical staff from WHO Primary Health Care Programme participated in a structured discussion of the ERPHC Framework with feedback emphasizing systems thinking, policy alignment and real-world feasibility.

Phase 2: External Review Group and WHO Structured Feedback + Action Planning (October 2025 to February 2026)

- In December 2025, the Community Readiness and Resilience Unit at WHO headquarters and RTSL set up an External Review Group of technical experts comprising programme implementers, academics and independent consultants, who, along with WHO technical teams selected from across all WHO regions provided consolidated structured feedback using a survey. A total of 35 completed surveys were received representing feedback from 40 people².
- The feedback was synthesized into major thematic areas and translated into a formal action plan for revision of the Framework and Operational Guide. Roles and responsibilities for WHO and RTSL teams were assigned with timelines.
- The ERPHC materials were pilot tested. For example, the ERPHC co-creation process in Nigeria brought together national and subnational stakeholders in a structured, participatory workshop to jointly analyse system performance, review evidence (including JEE findings and pilot lessons) and build consensus on priority interventions through iterative group work, prioritization exercises and validation discussions. Central to this process, the ERPHC self-assessment tool was used across the connect, detect, protect and treat capacities to assess current system performance, identify gaps and generate a shared analytical baseline, which directly informed prioritisation and the development of a feasible, high-impact intervention package. Feedback from the workshop was synthesized.
- This phase represented the most systematic and comprehensive consolidation of feedback.

Phase 3: Global Expert Consultation (February 2026, Cairo)

- A global expert hybrid consultation and validation meeting was held in Cairo, Egypt, on 25–26 February 2026. This meeting was attended by more than 40 members of the expert review group, WHO technical staff and RTSL. Participants reviewed the revised versions of the ERPHC Framework and Operational Guide, validating the strengths of the two documents and identifying remaining gaps and usability issues.
- Discussions during the Cairo validation workshop strengthened the ERPHC framework by improving conceptual clarity, ensuring consistent definitions and reinforcing an explicit all-hazards approach. It enhanced technical coherence by aligning capacities, outcomes and activities, while restructuring activities into clearer, output-based and health system-oriented components. The feedback also expanded the framework's technical completeness by incorporating missing elements. Overall, the process made the framework more integrated, evidence-aligned and adaptable to diverse real-world PHC contexts.

2 Some respondents submitted feedback as a group

Evidence synthesis

Evidence for the development and revision of the ERPHC framework and operational guide was gathered through a targeted, iterative literature search and synthesis process. Searches were conducted across peer-reviewed databases (e.g. PubMed, Global Health, Google Scholar) and grey literature sources (e.g. WHO, World Bank, the United Nations Children's Fund, the International Federation of the Red Cross and Red Crescent Societies and other organisation's repositories), using key terms related to primary health care, health security, epidemic preparedness, IPC, surveillance and service continuity. Relevant global frameworks and operational guidance were purposively selected based on relevance to PHC and emergency contexts, with additional snowballing from reference lists. Documents were screened for relevance to PHC-level implementation and inclusion of practical or evidence-based interventions and key findings were extracted and thematically synthesised across core capacities (connect, detect, protect, treat). This synthesis was continuously refined through triangulation with expert input, pilot country learning and multiple rounds of technical review (WHO team, the expert review group and global consultations) to ensure both scientific grounding and operational relevance. Guidance and resources related to humanitarian contexts, disaster risk management, public health and universal health coverage were also reviewed and incorporated as needed.

References

1. Frieden TR, Lee CT, Lamorde M, Nielsen M, McClelland A, Tangcharoensathien V. The road to achieving epidemic-ready primary health care. *The Lancet Public Health*. 2023;8(5):e383–e90 ([https://doi.org/10.1016/S2468-2667\(23\)00060-9](https://doi.org/10.1016/S2468-2667(23)00060-9)).
2. Mearns SL, Kamateeka M, Okwor T, Abba A, Obiekea C, Danjuma J et al. Progress in epidemic-ready primary health care: early pilot results from four African countries (Ethiopia, Nigeria, Sierra Leone and Uganda), December 2023 - October 2024. *BMJ Global Health*. 2025;10(9):e019249 (<https://doi.org/10.1136/bmjgh-2025-019249>).

Annex 8: ERPHC country planning and prioritization tool

The ERPHC country planning and prioritization tool is a practical tool to support country teams in applying the ERPHC framework and developing a tailored national ERPHC strategy. It helps identify strengths, gaps and priority results related to the ERPHC core capacities of **connect, detect, protect** and **treat**. In addition to a general contextual analysis, the assessment, planning and prioritization process guides countries in aligning their plans with ERPHC principles to strengthen emergency preparedness and response, service continuity and resilience across the health system.

Many ERPHC activities span multiple capacities, reflecting the integrated nature of PHC systems. Some activities are therefore repeated across capacities to illustrate these interconnected workflows. The activities listed are **illustrative rather than exhaustive** and should be adapted to national contexts—added, modified, prioritized, or removed based on system capacity, risks, geography and available resources. Countries are encouraged to define **context-appropriate minimum readiness packages** that enable PHC systems to detect, respond to and manage emergencies while maintaining essential services. To support implementation, activities are also grouped by **health system domains aligned with the WHO health system building blocks**, reinforcing that ERPHC strengthens core PHC system functions rather than creating a parallel system.

The assessment, planning and prioritization process should be completed in collaboration with various departments involved in emergency preparedness and response to ensure an accurate and comprehensive assessment. To prepare for the assessment ensure:

- all relevant team members are sufficiently briefed;
- source documents for required data are collected; and
- sufficient time is set aside for the assessment.

It is of utmost importance that the latest findings and recommendations from assessments such as Joint External Evaluation (JEE), States Parties Self-Assessment Annual Reports (SPAR) and 7-1-7 data are available and reviewed ahead of the self-assessment meeting. To complete the assessment, reflect on the questions on background/context and then score each assessment question against the categories provided in Table WA1.1 and provide a detailed narrative outlining the rationale behind the selection as well as any information on challenges or best practices. Instructions for scoring the level of country achievement using the planning and prioritization tool are outlined in table WA1.1 below.

Table WA1.1 Assessment categories

Level	Category	What it means in practice
1	not implemented	activity does not exist in practice. no structures, processes, or routine actions in place.
2	partially implemented	some elements exist (pilots, ad hoc practice, or limited geography), but not systematic, not routine, or not yet functional at scale.
3	implemented	activity is operational and routine in relevant settings, but quality, coverage, or consistency gaps remain.
4	fully implemented (with success / best practice)	activity is institutionalized, functioning effectively at scale, monitored and demonstrating good performance or innovation.

Once the assessment is done, review each section – **connect, detect, protect and treat** – to determine:

- the extent to which each area has been addressed (signalled by the extent to which capacities have been marked as green, orange and red);
- where significant challenges may lie; and
- which interventions within each capacity may need to be prioritized.

Table WA1.2 provides key questions to collect information about the general country context. It outlines data collection methods and sources used to analyse disease burden, public health risks and system readiness to inform ERPHC planning. It draws on established frameworks and evidence sources, including JEE (1, 2) and SPAR (3) findings to identify health security gaps, 7-1-7 performance metrics to highlight detection and response bottlenecks, and WHO benchmarks for health emergency capacities (4) to ensure alignment with global standards. Other information sources include Global Health Observatory (5) and Institute of Health Metrics and Evaluation (IHME) reports (6). It also incorporates relevant insights from IPC assessment Frameworks to assess facility-level readiness.

Table WA1.2 General country context for ERPHC implementation

Questions	Data collection methods	Sources of data	Comments
What diseases make up the main burdens of disease in this country? A. What are the most common outbreak-prone pathogens in this country? B. What are the potential outbreaks following natural disasters	- desktop search and document review - stakeholder interviews	- WHO country profiles (7) - The Global Health Observatory (5) - Country health ministry reports	
What public health emergencies are prevalent in this country?		- Country SDG progress reports - The Demographic and Health Surveys Program (reports by country) (8) - Institute of Health Metrics and Evaluation country profiles (6)	
What percentage of households have access to clean water and sanitation facilities (WASH)?		The Intergovernmental Panel on Climate Change sixth assessment report (9)	
What is the percentage of fully immunized children, who have received all recommended doses of all vaccines (including measles vaccine), according to the country EPI schedule?		UNICEF vaccination coverage data by country (10)	
What are the vaccination rates for each of the antigens in the country EPI schedule?			

Questions	Data collection methods	Sources of data	Comments
Which government departments are responsible for emergency preparedness and response coordination, surveillance, IPC, case management, RCCE, community health and PHC? Are there forums to coordinate activities/share information at national and local levels			
What are the key gaps from the country's most recent JEE or SPAR assessment related with PHC roles and community engagement?		• Joint External Evaluation tool: International Health Regulations (2005), 3rd ed. (1) • Joint External Evaluations (JEE) by country (2) • The Global Health Security (GHS) Index (11) • SPAR reports by country (3)	

EPI: expanded programme on immunization; GHSI: global health security index; IHME: Institute of Health Metrics and Evaluation; IPC: infection prevention and control; RCCE: risk communication and community engagement; SDG: Sustainable Development Goals; SPAR: States Parties Self-Assessment Annual Reporting; UNICEF: United Nations Children's Fund; WASH: water sanitation and hygiene promotion; WHO: World Health Organization.

Table WA1.3 is the ERPHC **connect** capacity country planning and prioritization tool. It provides a structured set of indicators, assessment questions and scoring criteria to assess CHW programmes, community leadership, RCCE capacities across PHC systems. Aligned with IHR core capacities, JEE and SPAR benchmarks, and informed by 7-1-7 bottlenecks related to early detection and response, it supports identification of gaps in RCCE, workforce readiness and community-system linkages. Key information sources include national health policies, national georeferenced CHW master lists and CHW registries. Planning and prioritisation using this tool ensures a comprehensive, multi-level assessment of how effectively country PHC systems connect with communities to support emergency readiness.

Table WA1.3 Connect capacity country planning and prioritization tool

Health system building block	#	Technical activity	Assessment questions	Data collection methods	Sources of data	Summary self-assessment	
						Assessment result*	Comments
Leadership and governance (Stewardship)	C1	RCCE focal points designated at PHC facility and district levels with defined roles for routine and emergency coordination with communities.	a. Have RCCE focal points been designated at the health facility level? b. Have RCCE focal points been designated at the district levels?	- desktop and document reviews - stakeholder Interviews	- information from stakeholders at national, subnational and PHC facility level (e.g. health facility managers) - focal point job descriptions		
	C2	Standard operating procedures for two-way communication and community engagement during emergencies - including who communicates what, to whom, through which channels and how feedback is escalated and acted upon - developed and disseminated.	a. Do SOPs on communication and community engagement exist? b. Have the SOPs been disseminated?	- desktop and document reviews - stakeholder Interviews	- information from stakeholders at national, subnational and PHC facility level (e.g. health facility managers) - SOPs/guidelines		
	C3	Formal mechanisms established for community representation in PHC emergency preparedness and response planning bodies and actions.	a. Has a formal mechanism for community representation been established? b. Are community leaders/ CHWs included in emergency preparedness and response planning bodies?	- desktop and document reviews - stakeholder Interviews	information from stakeholders at national, subnational and PHC facility level (e.g. health facility managers, other health workers and CHWs)		
	C4	Roles and capacities for community health emergency risk management defined, including risk assessment, planning and simulation exercises.	Have capacities for community health emergency risk management and assessment been developed?	- desktop and document reviews - stakeholder Interviews	- national georeferenced CHW master list and CHW registry - CHW strategy/ policy/ guidelines - information from stakeholders at national, subnational and sub-district level		

Health system building block	#	Technical activity	Assessment questions	Data collection methods	Sources of data	Summary self-assessment	
						Assessment result*	Comments
	C5	Multisectoral RCCE coordination SOPs formalized at community facing subnational level (health + disaster management + social welfare + education, as relevant) to support joint messaging and coordination during emergencies.	Have SOPs to support joint messaging and coordination during emergencies developed?	- desktop and document reviews - stakeholder Interviews	- information from stakeholders at national, subnational and PHC facility level (e.g. health facility managers, other health workers and CHWs) - SOPs/guidelines		
Health workforce (Staff)	C6	Health workers, including CHWs, engaged and mobilized as trusted influencers within communities.	Have health workers, including CHWs, been engaged to serve as trusted influencers within communities?	- desktop and document reviews - stakeholder Interviews	- information from stakeholders at national, subnational and PHC facility level (e.g. health facility managers, other health workers and CHWs) - CHW job descriptions		
	C7	Health workforce capacity in RCCE, including interpersonal communication skills, strengthened.	Has workforce capacity been built in RCCE, including interpersonal communication skills?	- desktop and document reviews - stakeholder Interviews	- information from stakeholders at national, subnational and PHC facility level (e.g. health facility managers, other health workers and CHWs) - global health observatory: health workforce (CHWs)		
	C8	Community health workforce capacity for health emergency management assessed and strengthened through training, regular supervision and referral linkages with PHC facilities.	a. Are there established processes for referrals, alert reporting and supervision that link CHWs with the broader health system/PHC health workers? b. Are there established training plans for CHWs?	- desktop and document reviews - stakeholder Interviews	- national georeferenced CHW master list and CHW registry - CHW strategy/ policy/ guidelines - information from stakeholders at national, subnational and PHC facility level (e.g. health facility managers, other health workers and CHWs)		

Health system building block	#	Technical activity	Assessment questions	Data collection methods	Sources of data	Summary self-assessment	
						Assessment result*	Comments
	C9	Community-based rapid response teams (RRTs) established and trained for community-based emergency detection and response.	Have community based rapid response teams been identified and trained on emergency detection and response?	- desktop and document reviews - stakeholder Interviews	- national georeferenced CHW master list and CHW registry - CHW strategy/ policy/ guidelines - information from stakeholders at sub-district and PHC facility level (e.g. health facility managers, other health workers and CHWs) - job descriptions for rapid response teams		
Service delivery (Services)	C10	Community structures, networks and stakeholders (e.g. CHWs, community leaders, faith-based organizations and civil society organizations) mapped and engaged, with community health committees established to enable collaboration and two-way communication with health services, local disaster committees and social welfare systems to support access to and use of health services.	a. Has mapping and engagement of community stakeholders been done? b. Are community health committees established?	- desktop and document reviews - stakeholder Interviews	- stakeholder analysis and mapping - country community health strategy - information from stakeholders at sub-district and PHC facility level (e.g. health facility managers, other health workers and CHWs) - terms of reference/ descriptions of community health committees		
	C11	Trusted messengers and preferred communication channels for key population groups, including marginalized groups, mapped and maintained in an updated contact list.	a. Have trusted messengers been identified? b. Have clear communication channels for key population groups mapped?	- desktop and document reviews - review results of stakeholder analysis and mapping	- information from stakeholders at sub-district and PHC facility level (e.g. health facility managers, other health workers and CHWs) - SOPs/guidelines		
	C12	Functional referral pathways/ systems between communities and PHC platforms reviewed and strengthened to support timely, coordinated and equitable access to health services.	Is there a functional referral system in place?	- desktop and document reviews - stakeholder Interviews	- referral policies - information from stakeholders at subnational, sub-district and PHC facility level (e.g. health facility managers, other health workers and CHWs)		

Health system building block	#	Technical activity	Assessment questions	Data collection methods	Sources of data	Summary self-assessment	
						Assessment result*	Comments
	C13	Routine community dialogue mechanisms (e.g. regular community forums, structured feedback sessions with CHWs/ community leaders) strengthened and linked to PHC facility action plan and local public health emergency preparedness and response plans.	a. Have routine dialogue mechanisms with CHWs/ community leaders been established and linked to PHC facility plans? b. Have routine dialogue mechanisms with CHWs/ community leaders been established and linked to local public health emergency preparedness and response plans?	desktop search and document reviews	- information from stakeholders at sub-district and PHC facility level (e.g. health facility managers, other health workers and CHWs) - SOPs/guidelines		
	C14	Mis/disinformation and rumour tracking integrated into existing feedback channels with processes for verification and message adaptation.	Has a system for mis/disinformation and rumour tracking been integrated into existing feedback channels?	desktop search and document reviews	- information from stakeholders at sub-district and PHC facility level (e.g. health facility managers, other health workers and CHWs) - SOPs/guidelines		
	C15	Communication and coordination mechanisms between local public health offices, PHC facilities and CHWs strengthened.	Is there a communication and coordination mechanism established between local public health offices, PHC facilities and CHWs?	desktop search and document reviews	- information from stakeholders at sub-district and PHC facility level (e.g. health facility managers, other health workers and CHWs) - SOPs/guidelines		
	C16	Community health literacy levels assessed and interventions co-created to improve understanding of preventive measures (including vaccinations), symptoms and reporting.	a. Have community health literacy levels been assessed? b. Have preventive community outreach interventions been co-created?	- desktop and document reviews - stakeholder interviews	- information from stakeholders at sub-district and PHC facility level (e.g. health facility managers, other health workers and CHWs) - global health observatory: health workforce (CHWs)		

Health system building block	#	Technical activity	Assessment questions	Data collection methods	Sources of data	Summary self-assessment	
						Assessment result*	Comments
	C17	Communities are mobilized and trained to undertake preventive and anticipatory actions before, during and after seasonal disasters and imminent threats.	Have communities been mobilized to conduct anticipatory action before, during or after seasonal disasters?	- desktop and document reviews - stakeholder interviews	- national georeferenced CHW master list and CHW registry - CHW strategy/ policy/ guidelines - information from stakeholders at sub-district and PHC facility level (e.g. health facility managers, other health workers and CHWs)		
Health information systems (Systems)	C18	Functional systems and platforms (digital and non-digital) established to capture, share and analyse community data and feedback within health information systems.	a. Is there a functioning health information system for community feedback? b. Is there a functional system to manage data from the community?	- desktop and document reviews - stakeholder interviews	- information from stakeholders at national, subnational and PHC facility level (e.g. health facility managers, other health workers and CHWs) - global Health Observatory: health Workforce (CHWs)		
	C19	Digital and mobile health platforms established to support community access to health information and digital literacy.	Are digital and mobile health platforms used and strengthened to support community access to health information?	- desktop and document reviews - stakeholder interviews	- information from stakeholders at national, subnational and PHC facility level (e.g. health facility managers, other health workers and CHWs) - global health observatory: health workforce (CHWs)		
	C20	Inter-community information sharing and collaboration mechanisms established or strengthened to support timely and effective emergency preparedness and response. Cross-border collaboration established where necessary.	Are cross-border and inter-community information sharing and collaboration mechanisms in place?	- desktop and document reviews - stakeholder interviews	information from stakeholders at national, subnational, sub-district and PHC facility level (e.g. health facility managers, other health workers and CHWs)		

Health system building block	#	Technical activity	Assessment questions	Data collection methods	Sources of data	Summary self-assessment	
						Assessment result*	Comments
	C21	Social listening mechanisms (e.g. hotline, complaint systems, social media listening, digital platforms) established to detect concerns, manage infodemics and support responsive communication.	Are systems for gathering community feedback and facilitating information exchange between health facilities and community members in place?	- desktop and document reviews - stakeholder interviews	- information from stakeholders at national, subnational and PHC facility level (e.g. health facility managers, other health workers and CHWs) - global health observatory: health workforce (CHWs)		
	C22	Operational systems established for continuous generation and use of community evidence to inform interventions, communication strategies and trust monitoring linked to RCCE adaptations.	Have operational systems for continuous generation and use of BCI been established?	- desktop and document reviews - stakeholder interviews	information from stakeholders at national, subnational and PHC facility level (e.g. health facility managers, other health workers and CHWs)		
	C23	Accountability mechanisms which link feedback to action institutionalized to report back to communities on actions taken in response to community feedback.	Have mechanisms been established and institutionalized to report back to communities?	- desktop and document reviews - stakeholder interviews	information from stakeholders at national, subnational and PHC facility level (e.g. health facility managers, other health workers and CHWs)		
	C24	Formal intersectoral data-sharing agreements and one health coordination established at the community and local levels to support early detection of and response to climate-sensitive and emerging zoonotic origin health threats.	Are data sharing agreements with other sectors in place?	- desktop and document reviews - stakeholder interviews	- national georeferenced CHW master list and CHW registry - CHW strategy/ policy/ guidelines - information from stakeholders at sub-district and PHC facility level (e.g. health facility managers, other health workers and CHWs)		
	C25	Routine inter- and after-action reviews conducted with PHC professionals, CHWs and community representatives following emergency responses.	Are inter- and after-action reviews routinely conducted with health workers and community representatives following emergency responses?	- desktop and document reviews - stakeholder interviews	- CHW strategy/ policy/ guidelines - information from stakeholders at sub-district and PHC facility level (e.g. health facility managers, other health workers and CHWs)		

CHWs: community health worker; PHC: primary health care; RCCE: risk communication and community engagement; SOP: standard operating procedure

* 1. not implemented, 2. partially implemented, 3. implemented, 4. fully implemented (with success / best practice)

Summary [connect]

Based on the scoring done above, please outline the key strengths of your programme	
• What are the core gaps your programme needs to address? • What are the gaps identified through the country JEE, IHR and Infection Prevention and Control Assessment Framework? • Why do these core gaps exist?	
• What are the critical priorities for your programme? • What are the key priority areas based on the gaps identified through the country JEE/7-1-7 bottlenecks, IHR and Infection Prevention and Control Assessment Framework? • What strategies will the country use to address identified priority areas? • Which stakeholders will you work with to address identified priorities?	

Table WA1.4 is the **detect** capacity country planning and prioritisation tool. It presents a structured set of technical activities, assessment questions and scoring criteria to assess surveillance and early detection capacities within PHC systems. Aligned with IHR core capacities and informed by JEE, SPAR and IDSR frameworks, it helps identify gaps in event detection, reporting, specimen referral, community-based surveillance and data use across PHC and community levels. It also reflects lessons from 7-1-7 bottlenecks related to timely detection, notification and early response, supporting a practical assessment of how well PHC systems are able to detect and escalate priority health threats. Sources of information include country IDSR policies and guidelines, service availability and readiness assessment indicators and information from stakeholders.

Table WA1.4 Detect capacity country planning and prioritisation tool

Health system building block	#	Technical activity	Assessment questions	Data collection methods	Sources of data	Summary self-assessment	
						Assessment result*	Comments
Leadership and governance (Stewardship)	D1	Standard operating procedures and guidelines for surveillance of priority health events, diseases and conditions at PHC facilities developed and disseminated.	a. Have SOPs and surveillance guidelines been developed? b. Have SOPs and surveillance guidelines been disseminated to PHC facilities?	- desktop search and document reviews - stakeholder interviews	- country IDSR strategy/ policy/ guidelines - IDSR data at health facility level - document on roles and responsibilities (job descriptions) for IDSR at PHC level - country IDSR reporting system - guidelines for IDSR in Africa - indicators for monitoring performance of core functions of integrated disease surveillance and response - information from stakeholders at national, subnational, sub-district and PHC facility level (e.g. health facility managers, other health workers and CHWs)		
Health workforce (Staff)	D2	Surveillance focal points designated at PHC facilities and provided with training and capacity strengthening to coordinate surveillance activities.	a. Have surveillance focal points been designated? b. Have surveillance focal points been trained?	- desktop search and document reviews - stakeholder interviews	- country IDSR strategy/ policy/ guidelines - IDSR data at health facility level - document on roles and responsibilities (job descriptions) for IDSR at PHC level - country IDSR reporting system - guidelines for IDSR in Africa - indicators for monitoring performance of core functions of integrated disease surveillance and response - information from stakeholders at sub-district and PHC facility level (e.g. health facility managers, other health workers and CHWs)		

Health system building block	#	Technical activity	Assessment questions	Data collection methods	Sources of data	Summary self-assessment	
						Assessment result*	Comments
	D3	PHC health worker capacity strengthened for detection and reporting of priority events, diseases and conditions through training, mentorship and integration into continuing professional development.	Has workforce capacity been strengthened to detect and report priority diseases and conditions?	- desktop search and document reviews - stakeholder interviews	- country IDSR strategy/ policy/ guidelines - IDSR data at health facility level - document on roles and responsibilities (job descriptions) for IDSR at PHC level - country IDSR reporting system - guidelines for IDSR in Africa - indicators for monitoring performance of core functions of integrated disease surveillance and response - information from stakeholders at national, subnational, sub-district and PHC facility level (e.g. health facility managers, other health workers and CHWs)		
	D4	CHW capacity strengthened to detect and report priority events and unusual health events at community level.	Have CHWs received training on detection and reporting of priority events and unusual health events?	- desktop and document reviews - stakeholder interviews	- surveillance system guidelines for the health workers and CHWs - information from stakeholders at sub-district and PHC facility level (e.g. health facility managers, other health workers and CHWs)		
	D5	Health worker capacity strengthened in specimen collection, packaging, storage and safe transport for suspected priority diseases.	Has healthcare worker capacity been built to collect, package, store and transport specimens?	- desktop and document reviews - stakeholder interviews	- surveillance system guidelines for health workers and CHWs - information from stakeholders at subnational, sub-district and PHC facility level (e.g. health facility managers, other health workers and CHWs)		

Health system building block	#	Technical activity	Assessment questions	Data collection methods	Sources of data	Summary self-assessment	
						Assessment result*	Comments
Service delivery (Services)	D6	Health risks at PHC and subnational levels mapped and prioritized to guide surveillance activities and align investments in human resources, processes, resources and systems.	a. Have health risks at the PHC level been mapped and prioritized to guide surveillance activities? b. Have health risks at subnational levels been mapped and prioritized to guide surveillance activities?	- desktop and document reviews - stakeholder interviews	- surveillance system guidelines for the health workers and CHWs - information from stakeholders at subnational, sub-district and PHC facility level (e.g. health facility managers, other health workers and CHWs) - country IDSR strategy/ policy/ guidelines - IDSR data at health facility level		
	D7	Surveillance capacity of civil society organizations (CSOs) and non-governmental actors mapped and integrated into local and national surveillance system through PHC service platforms to support early detection and reporting of priority health events from communities.	a. Has surveillance capacity been built among CSOs and non-governmental actors? b. Has surveillance capacity among CSOs and non-governmental actors been mapped and integrated into local and national surveillance systems?	- desktop and document reviews - stakeholder interviews	information from stakeholders at subnational, sub-district and PHC facility level (e.g. health facility managers, other health workers and CHWs)		
	D8	Functional referral systems established between primary, secondary and tertiary care levels with effective feedback mechanisms.	Is a functional referral system in place between all levels that includes feedback mechanisms?	- stakeholder interviews - desktop search and document reviews	- referral policies - information from stakeholders at subnational, sub-district and PHC facility level (e.g. health facility managers, other health workers and CHWs)		
	D9	One health surveillance coordination mechanisms formalized to link public health surveillance and response functions at PHC levels with equivalent level of animal health and environmental health sectors.	Has one health coordination been formalized?	- stakeholder interviews - desktop search and document reviews	information from stakeholders at national and subnational level		

Health system building block	#	Technical activity	Assessment questions	Data collection methods	Sources of data	Summary self-assessment	
						Assessment result*	Comments
	D10	Basic population health data monitoring frameworks developed and implemented to support early detection of changes in mortality and morbidity trends for timely identification of health threats in communities.	Are basic population health data monitoring frameworks implemented?	- desktop search and document reviews - stakeholder interviews	- information from stakeholders at national, subnational and local government level - country IDSR strategy/ policy/ guidelines		
	D11	Joint risk and preparedness assessments conducted with key sectors and partners engaged in emergency management (e.g. disaster management and meteorological services) at PHC levels.	Have joint risk and preparedness assessments been conducted at the PHC level with key sectors?	- desktop search and document reviews - stakeholder interviews	- information from stakeholders at national, subnational, sub-district and PHC facility level (e.g. health facility managers, other health workers and CHWs) - assessment reports		
Medicines, Medical Products, Vaccines and Technologies (Supplies)	D12	Point-of-care diagnostics and supplies for specimen collection and transportation made available at PHC facilities.	a. Are point-of-care diagnostics and supplies for specimen collection available at PHC facilities? b. Are systems and resources in place to transport specimens?	- desktop and document reviews - stakeholder interviews	- surveillance system guidelines for the health workers and CHWs - information from stakeholders at subnational, sub-district and PHC facility level (e.g. health facility managers, other health workers and CHWs) - service availability and readiness assessment indicators		
	D13	Laboratory networks and specimen collection and transportation mechanisms established to link PHC facilities with testing laboratories for priority diseases.	Are networks and sample collection mechanisms linking PHC facilities to laboratories in place?	- desktop and document reviews - stakeholder interviews	- surveillance system guidelines for the health workers and CHWs - information from stakeholders at subnational, sub-district and PHC facility level (e.g. health facility managers, other health workers and CHWs)		

Health system building block	#	Technical activity	Assessment questions	Data collection methods	Sources of data	Summary self-assessment	
						Assessment result*	Comments
Health information systems (Systems)	D14	Functional communication channels and data collection and feedback mechanisms established between PHC facilities and community facing public health levels, including coordination with local disaster response teams.	Have functional communication channels and feedback loops been established?	- desktop and document reviews - stakeholder interviews	- health information system strategy/ policy/ guideline documents - surveillance system guidelines for the health workers and CHWs - information from stakeholders at national, subnational, sub-district and PHC facility level (e.g. health facility managers, other health workers and CHWs) - information from managers, MEL lead, HIS lead at PHC facility, etc.		
	D15	Routine data quality assurance and analysis conducted at sub-district level, with structured feedback provided to PHC facilities.	Is regular data quality assurance and routine data analysis conducted at sub-district level?	- desktop and document reviews - stakeholder interviews	information from stakeholders at sub-district and PHC facility level (e.g. health facility managers, other health workers and CHWs)		
	D16	Functioning health information systems established, including digital approaches where feasible, to collect, analyse and report surveillance data from health facilities and communities (community-based surveillance).	a. Is there a functioning health information system for PHC facility surveillance data? b. Is there a functioning health information system for community level surveillance data?	- desktop and document reviews - stakeholder interviews	- health information system strategy/ policy/guideline documents - surveillance system guidelines for the health workers and CHWs - information from stakeholders at national, subnational, sub-district and PHC facility level (e.g. health facility managers, other health workers and CHWs) - information from managers, MEL lead, HIS lead at PHC facility, etc.		
	D17	Rapid risk and vulnerability assessments conducted during outbreaks, including analysis of gender and protection risks, particularly in conflict-affected areas.	Have rapid risk and vulnerability assessments been conducted?	- desktop and document reviews - stakeholder interviews	information from stakeholders at subnational, sub-district and PHC facility level (e.g. health facility managers, other health workers and CHWs)		

Health system building block	#	Technical activity	Assessment questions	Data collection methods	Sources of data	Summary self-assessment	
						Assessment result*	Comments
	D18	Community event-based surveillance systems and reporting mechanisms established including hotlines and trained community informants.	Are community EBS and reporting mechanisms in place?	- desktop and document reviews - stakeholder interviews	- health information system strategy/ policy/guideline documents - surveillance system guidelines for the health workers and CHWs - information from stakeholders at national, subnational, sub-district and PHC facility level (e.g. health facility managers, other health workers and CHWs) - information from managers, MEL lead, HIS lead at PHC facility, etc.		
	D19	Early warning systems established that integrate health data with data from other sectors to enable PHC facilities to monitor risk drivers, anticipate health threats and mitigate impacts.	Are early warning systems in place?	- desktop search and document reviews - stakeholder interviews	- country IDSR strategy/ policy/ guidelines - IDSR data at health facility level - country IDSR reporting system - guidelines for WHO IDSR in Africa - indicators for monitoring performance of core functions of integrated disease surveillance and response - information from stakeholders at national, subnational, sub-district and PHC facility level (e.g. health facility managers, other health workers and CHWs)		
	D20	Surveillance systems digitally integrated to transition from fragmented or paper-based approaches to interoperable digital platforms.	Has digital integration of surveillance systems been completed?	- desktop and document reviews - stakeholder interviews	- health information system strategy/ policy/guideline documents - surveillance system guidelines for the health workers and CHWs - information from stakeholders at national, subnational, sub-district and PHC facility level (e.g. health facility managers, other health workers and CHWs) - information from managers, MEL lead, HIS lead at PHC facility, etc.		

Health system building block	#	Technical activity	Assessment questions	Data collection methods	Sources of data	Summary self-assessment	
						Assessment result*	Comments
	D21	Environmental surveillance systems established, including wastewater monitoring where appropriate to the context.	Are environmental surveillance systems established?	- desktop search and document reviews - stakeholder interviews	- country IDSR strategy/ policy/ guidelines - guidelines for WHO IDSR in Africa - information from stakeholders at national, subnational and local government level		

CHWs: community health workers; CSOs: civil society organizations; IDSR: integrated disease surveillance and response; HIS: health information system; MEL: measurement, evaluation and learning; PHC: primary health care; WHO: World Health Organization

* 1. not implemented, 2. partially implemented, 3. implemented, 4. fully implemented (with success / best practice)

Summary [detect]

Based on the scoring done above, please outline the key strengths of your programme	
- What are the core gaps your programme needs to address? - What are the gaps identified through the country JEE/7-1-7 bottlenecks, IHR and Infection Prevention and Control Assessment Framework? - Why do these core gaps exist?	
- What are the critical priorities for your programme? - What are the key priority areas based on the gaps identified through the country JEE/7-1-7 bottlenecks, IHR and Infection Prevention and Control Assessment Framework? - What strategies will the country use to address identified priority areas? - Which stakeholders will you work with to address identified priorities?	

Table WA1.5 is the **protect** capacity country planning and prioritization tool. It presents a structured set of indicators, assessment questions and scoring criteria to assess the extent to which PHC systems are able to protect health workers, patients and communities during routine service delivery and health emergencies. Aligned with relevant IHR core capacities and informed by JEE and SPAR findings, it supports identification of gaps in immunization, IPC, WASH, antimicrobial stewardship, health worker protection, safe patient flow and supply systems. It also draws on relevant IPC assessment frameworks and lessons from 7-1-7 bottlenecks related to delayed protection measures and facility readiness, helping countries assess how well PHC platforms can reduce transmission risks and maintain safe, resilient services during emergencies. Key sources of information include regular IPC assessments, SARA Indicators and Harmonized Health Facility Assessment (HHFA) Indicator Inventory

Table WA1.5 Protect capacity country planning and prioritization tool

Health system building block	#	Technical activity	Assessment questions	Data collection methods	Sources of data	Summary self-assessment	
						Assessment result*	Comments
Leadership and governance (Stewardship)	P1	Standard operating procedures and guidelines for routine and emergency immunization developed and disseminated to PHC facilities.	a. Have SOPs and guidelines for routine and emergency immunization at PHC facilities been developed? b. Have SOPs and guidelines for routine and emergency immunization at PHC facilities been disseminated to PHC facilities?	- desktop and document reviews - stakeholder interviews	- information from stakeholders at national, subnational, sub-district and PHC facility level (e.g. health facility managers, other health workers and CHWs) - national SOPs/guidelines		
	P2	Functional IPC and WASH committees established and maintained to support implementation and oversight of IPC and WASH activities at PHC facilities.	a. Are IPC committees in place at PHC facilities? b. Are WASH committees in place at PHC facilities?	- desktop and document reviews - stakeholder interviews	- IPC guidelines and processes - WASH guidelines and processes - PHC facility staffing lists - information from stakeholders at sub-district and PHC facility level (e.g. health facility managers, other health workers and CHWs)		
	P3	Standard operating procedures and guidelines for IPC and WASH developed and disseminated to PHC facilities.	a. Are IPC SOPs and operational guidelines developed and disseminated at PHC facilities? b. Are WASH SOPs and operational guidelines developed and disseminated at PHC facilities?	- desktop and document reviews - stakeholder interviews	- IPC guidelines and processes - WASH guidelines and processes - IPC and WASH training curriculum and materials - IPC and WASH assessment guidelines and tools in use by health facility - AMS implementation guidelines and processes - harmonized health facility assessment indicator inventory - information from stakeholders at sub-district and PHC facility level (e.g. health facility managers, other health workers and CHWs)		

Health system building block	#	Technical activity	Assessment questions	Data collection methods	Sources of data	Summary self-assessment	
						Assessment result*	Comments
Health workforce (Staff)	P4	health worker capacity strengthened in vaccine storage, transport, deployment and administration in accordance with immunization standard operating procedures.	c. Has workforce capacity been built to store, transport and deploy vaccines according to immunization SOPs?	- desktop and document reviews - stakeholder interviews	information from stakeholders at sub-district and PHC facility level (e.g. health facility managers, other health workers and CHWs)		
	P5	IPC and WASH focal points designated at PHC facilities to coordinate implementation and oversight of IPC and WASH activities.	a. Are IPC focal points designated and trained at PHC facilities? b. Are WASH focal points designated and trained at PHC facilities?	- desktop and document reviews - stakeholder interviews	- IPC guidelines and processes - WASH guidelines and processes - PHC facility staffing lists - focal point job descriptions - IPC and WASH training curriculum and materials - assessments of health worker knowledge and skills on IPC and WASH - review of IPC and WASH assessment results (PHC) - knowledge attitudes and practices assessment among health staff to establish a culture of IPC - IPC assessment guidelines and tools in use by health facility - WASH assessment guidelines and tools in use by health facility - AMS implementation guidelines and processes - harmonized health facility assessment indicator inventory - information from stakeholders at sub-district and PHC facility level (e.g. health facility managers, other health workers and CHWs)		

Health system building block	#	Technical activity	Assessment questions	Data collection methods	Sources of data	Summary self-assessment	
						Assessment result*	Comments
	P6	health worker capacity strengthened in IPC, antimicrobial resistance (AMR) and WASH through training, mentorship and supportive supervision mechanisms.	a. Has workforce capacity been built in IPC and AMR? b. Has workforce capacity been built for WASH? c. Is there a system for ongoing mentorship and supervision?	- desktop and document reviews - stakeholder interviews	- IPC training curriculum and materials - WASH training curriculum and materials - assessments of health worker knowledge and skills on IPC and WASH - review of IPC assessment results (PHC) - review of WASH assessment results (PHC) - knowledge attitudes and practices assessment among health staff to establish a “culture of IPC” - information from stakeholders at sub-district and PHC facility level (e.g. health facility managers, other health workers and CHWs)		
	P7	PHC facilities equipped to operate as safe and protected environments for patients and health workers, including provision of shelters, secure areas and protective measures for health workers.	a. Are PHC facilities equipped to operate as safe and protected environments for patients and health workers? b. Are secure areas/shelters in place? c. Are protective measures in place for health workers?	- desktop and document reviews - stakeholder interviews	- review of IPC assessment results (PHC) - review of WASH assessment results (PHC) - information from stakeholders at sub-district and PHC facility level (e.g. health facility managers, other health workers and CHWs) - service availability and readiness assessment indicators - harmonized health facility assessment indicator inventory		
	P8	Systems established to ensure availability of psychosocial first aid and mental health and psychosocial support (MHPSS) for health workers, including referral pathways for additional support.	Are systems in place to ensure psychosocial protection of health workers?	- desktop and document reviews - stakeholder interviews	information from stakeholders at subnational, sub-district and PHC facility level (e.g. health facility managers, other health workers and CHWs)		

Health system building block	#	Technical activity	Assessment questions	Data collection methods	Sources of data	Summary self-assessment	
						Assessment result*	Comments
Service delivery (Services)	P9	Systems established in PHC facilities to reduce infection risk through safe patient flow, including screening, hand hygiene, ventilation, sanitation, routine cleaning, optimized consultation spaces and overflow or referral mechanisms when capacity is reached.	a. Are screening, triage and isolation systems in place at PHC facilities? b. Are systems in place for patient flow? c. Are systems in place for routine cleaning and sanitation?	- desktop and document reviews - stakeholder interviews	- service availability and readiness assessment indicators - harmonized health facility assessment indicator inventory - information from stakeholders at sub-district and PHC facility level (e.g. health facility managers, other health workers and CHWs) - review of IPC assessment results (PHC) - review of WASH assessment results (PHC)		
	P10	Screening, triage and isolation systems and infrastructure operationalized at PHC facilities, including overflow and sanitation capacity to safely manage outbreaks, mass-casualty events and infectious patients.	Are systems and infrastructure for screening, triage and isolation in place at PHC facilities?	- desktop and document reviews - stakeholder interviews	- IPC guidelines and processes - harmonized health facility assessment indicator inventory - information from stakeholders at sub-district and PHC facility level (e.g. health facility managers, other health workers and CHWs) - review of IPC assessment results (PHC)		
	P11	Annual micro-plans developed for routine and emergency vaccination services to guide planning, resource allocation and implementation.	Are annual micro-plans in place for routine and emergency vaccination services?	- desktop and document reviews - stakeholder interviews	- information from stakeholders at national, subnational, sub-district and PHC facility level (e.g. health facility managers, other health workers and CHWs) - plans/SOPs on vaccination		
	P12	PHC facilities have climate-resilient and emergency ready infrastructure for WASH, energy, power supply and medical waste management.	a. Has climate resilient and emergency ready infrastructure planning been implemented? b. Is emergency and climate resilient health facility infrastructure in place?	- desktop and document reviews - stakeholder interviews	- information from stakeholders at subnational, sub-district and PHC facility level (e.g. health facility managers, other health workers and CHWs) - plans/SOPs on vaccination		

Health system building block	#	Technical activity	Assessment questions	Data collection methods	Sources of data	Summary self-assessment	
						Assessment result*	Comments
	P13	Basic WASH services are available and functional in health facilities and communities to support safe care and infection prevention.	a. Are basic WASH services available at PHC facilities? b. Are basic WASH services functional at PHC facilities?	- desktop and document reviews - stakeholder interviews	- service availability and readiness assessment indicators - harmonized health facility assessment indicator inventory - global report on infection prevention and control (SPAR indicators) - information from stakeholders at sub-district and PHC facility level (e.g. health facility managers, other health workers and CHWs)		
	P14	Antimicrobial stewardship (AMS) activities implemented at PHC facilities to promote appropriate antimicrobial use and reduce antimicrobial resistance.	Are AMS activities implemented at PHC facilities?	- desktop and document reviews - stakeholder interviews	information from stakeholders at subnational, sub-district and PHC facility level (e.g. health facility managers, other health workers and CHWs)		
	P15	IPC measures integrated into referral and pre-hospital transfer systems, including ambulance services, to safely manage patients requiring transmission-based precautions.	Have IPC measures been integrated into referral and pre-hospital transfer systems?	- desktop and document reviews - stakeholder interviews	- information from stakeholders at subnational, sub-district and PHC facility level (e.g. health facility managers, other health workers and CHWs) - IPC guidelines and processes - review of IPC assessment results (PHC)		
Medicines, medical products, vaccines and technologies (Supplies)	P16	Functional cold chain infrastructure established and maintained with reliable vaccine stocks at PHC facilities to support routine and emergency immunization services.	a. Is functional cold chain infrastructure in place at PHC facilities? b. Are vaccines reliably available at PHC facilities?	- desktop and document reviews - stakeholder interviews	- service availability and readiness assessment indicators - harmonized health facility assessment indicator inventory - information from stakeholders at sub-district and PHC facility level (e.g. health facility managers, other health workers and CHWs)		

Health system building block	#	Technical activity	Assessment questions	Data collection methods	Sources of data	Summary self-assessment	
						Assessment result*	Comments
	P17	Essential IPC supplies and equipment and vaccines are available at PHC facilities through strengthened supply chains and responsive stock management systems aligned with minimum standards and SOPs.	a. Are stock management systems in place at PHC facilities for uninterrupted essential IPC supplies, equipment and vaccines? b. Are essential PPE and other IPC supplies available at PHC facilities? c. Have the cold chain and stock management systems strengthened for uninterrupted supply of vaccines?	- desktop and document reviews - stakeholder interviews	- service availability and readiness assessment indicators - harmonized health facility assessment indicator inventory - global report on infection prevention and control (SPAR indicators) - information from stakeholders at sub-district and PHC facility level (e.g. health facility managers, other health workers and CHWs)		
Health information systems (Systems)	P18	Regular IPC monitoring conducted using standardized assessment tools with corrective action plans implemented to support continuous quality improvement.	Is regular IPC monitoring conducted with implementation of corrective action plans?	- desktop and document reviews - stakeholder interviews	- IPC guidelines and processes - review of IPC assessment results (PHC) - IPC assessment guidelines and tools in use by health facility - service availability and readiness assessment indicators - harmonized health facility assessment indicator inventory - global report on infection prevention and control (SPAR indicators) - information from stakeholders at sub-district and PHC facility level (e.g. health facility managers, other health workers and CHWs)		
	P19	Intra-action and after-action reviews and simulation exercises conducted regularly to assess and strengthen system readiness for health emergencies.	a. Are intra-action and after reviews conducted at PHC facilities? b. Are simulation exercises conducted regularly?	- desktop and document reviews - stakeholder interviews	information from stakeholders at sub-district and PHC facility level (e.g. health facility managers, other health workers and CHWs)		

AMR: antimicrobial resistance; AMS: antimicrobial stewardship; CHWs: community health worker; IPC: infection prevention and control; PHC: primary health care; SOP: standard operating procedure; SPAR: Self-Assessment Annual Reporting; WASH: water, sanitation and hygiene

* 1. not implemented, 2. partially implemented, 3. implemented, 4. fully implemented (with success / best practice)

Summary [protect]

Based on the scoring done above, please outline the key strengths of your programme	
What are the core gaps your programme needs to address? What are the gaps identified through the country JEE/7-1-7 bottlenecks, IHR and IPC assessment framework? Why do these core gaps exist?	
What are the critical priorities for your programme? What are the key priority areas based on the gaps identified through the country JEE/7-1-7 bottlenecks, IHR and Infection Prevention and Control Assessment Framework? What strategies will the country use to address identified priority areas? Which stakeholders will you work with to address identified priorities?	

IHR: International Health Regulations; IPC: infection prevention and control; JEE: Joint External Evaluation.

Table WA1.6 is the **treat** capacity country planning and prioritization tool. It presents a structured set of indicators, assessment questions and scoring criteria to assess the capacity of PHC systems to deliver treatment, maintain continuity of essential services and manage surges during health emergencies. Aligned with relevant IHR core capacities and informed by JEE and SPAR findings, it supports identification of gaps in emergency case management, referral systems, service continuity planning, surge workforce arrangements, essential supplies and health information systems. It also reflects lessons from 7-1-7 bottlenecks related to delayed response and continuity of care, helping countries assess how well PHC platforms can provide timely, safe and sustained care during outbreaks, disasters and other acute shocks. Key information sources include information from relevant stakeholders, country policies and guidelines, service availability and readiness assessment indicators, and the harmonized health facility assessment indicator inventory.

Table WA1.6 Treat capacity country planning and prioritization tool

Health system building block	#	Technical activity	Assessment questions	Data collection methods	Sources of data	Summary self-assessment	
						Assessment result*	Comments
Leadership and Governance (Stewardship)	T1	National policies, guidelines and standard operating procedures for emergency case management and continuity of essential services at PHC and community levels developed and disseminated.	a. Have SOPs for emergency case management and service continuity been developed? b. Have SOPs for emergency case management and service continuity been disseminated to PHC facilities?	- desktop and document reviews - stakeholder interviews	- service availability and readiness assessment indicators - harmonized health facility assessment indicator inventory - information from stakeholders at national, subnational, sub-district and PHC facility level (e.g. health facility managers, other health workers and CHWs)		
	T2	Standard operating procedures for community health workers to guide diagnosis, treatment and follow-up for iCCM, chronic care and RMNCAH services developed and disseminated.	a. Have SOPs for CHWs to guide diagnosis, treatment and follow-up for iCCM, chronic care and RMNCAH services been developed? b. Have SOPs for CHWs to guide diagnosis, treatment and follow-up for iCCM, chronic care and RMNCAH services been disseminated to PHC facilities?	- desktop and document reviews - stakeholder interviews	- service availability and readiness assessment indicators - harmonized health facility assessment indicator inventory - information from stakeholders at national, subnational, sub-district and PHC facility level (e.g. health facility managers, other health workers and CHWs)		
	T3	Routine supportive supervision systems in place for services delivered at community and PHC facility levels.	a. Are routine supportive supervision systems in place for the community level? b. Are routine supportive supervision systems in place for the PHC facility level?	- desktop and document reviews - stakeholder interviews	information from stakeholders at national, subnational, sub-district and PHC facility level (e.g. health facility managers, other health workers and CHWs)		
	T4	Regular simulation exercises conducted to test PHC readiness, service continuity plans and surge response capacity.	Are regular simulation exercises conducted for PHC readiness, service continuity and surge response capacity?	- desktop and document reviews - stakeholder interviews	- information from stakeholders at subnational, sub-district and PHC facility level (e.g. health facility managers, other health workers and CHWs) - simulation exercise reports		

Health system building block	#	Technical activity	Assessment questions	Data collection methods	Sources of data	Summary self-assessment	
						Assessment result*	Comments
	T5	Subnational disaster management coordination mechanisms established to link PHC facilities, community rapid response teams, ambulance services and national disaster response systems led by the disaster management ministry.	a. Are subnational disaster management coordination mechanisms established to link PHC facility level, community rapid response teams, ambulance services and national disaster response systems led by the disaster management ministry? b. Is there a coordination mechanism between PHC facilities and emergency response systems?	- desktop and document reviews - stakeholder interviews	information from stakeholders at sub-district and PHC facility level (e.g. health facility managers, other health workers and CHWs)		
Health workforce (Staff)	T6	Functional community health worker (CHW) networks in place to support treatment and service delivery during emergencies, including iCCM, chronic disease follow-up and maternal and child health services.	Are functional CHW networks in place to support treatment and service delivery (iCCM, chronic disease follow-up and maternal and child health) during emergencies?	- desktop and document reviews - stakeholder interviews	- information from stakeholders at sub-district and PHC facility level (e.g. health facility managers, other health workers and CHWs) - job descriptions for CHWs		
	T7	Surge workforce deployment mechanisms established and maintained, including preparedness planning and roster systems to mobilize additional personnel during emergencies.	Has surge capacity been established for workforce deployment?	- desktop and document reviews - stakeholder interviews	- information from stakeholders at sub-district and PHC facility level (e.g. health facility managers, other health workers and CHWs) - health worker rosters with roles and responsibilities		

Health system building block	#	Technical activity	Assessment questions	Data collection methods	Sources of data	Summary self-assessment	
						Assessment result*	Comments
	T8	Structured training and mentorship systems established for PHC and CHW workforces to strengthen emergency preparedness, service continuity planning and response to diverse hazards.	a. Is structured training for PHC and CHW workforces in place? b. Is structured mentorship in place for PHC and CHW workforces in place?	- desktop and document reviews - stakeholder interviews	- information from stakeholders at national, subnational, sub-district and PHC facility level (e.g. health facility managers, other health workers and CHWs) - training curricula		
	T9	PHC health worker and CHW capacity strengthened for case management of outbreaks, climate-related health conditions and mass casualty events through training, ongoing supervision and skills development.	Has capacity been strengthened for case management during outbreaks, climate-related shocks and mass casualty events?	- desktop and document reviews - stakeholder interviews	information from stakeholders at national, subnational, sub-district and PHC facility level (e.g. health facility managers, other health workers and CHWs)		
Service delivery (Services)	T10	Functional referral pathways and patient transportation mechanisms established to ensure end-to-end clinical care from community level through primary, secondary and tertiary care.	a. Are functional referral pathways in place? b. Are functional transport mechanisms in place?	- desktop and document reviews - stakeholder interviews	- referral policies - service availability and readiness assessment indicators - harmonized health facility assessment indicator inventory - information from stakeholders at national, subnational, sub-district and PHC facility level (e.g. health facility managers, other health workers and CHWs)		
	T11	Screening and triage systems and infrastructure operationalized within PHC facilities.	Are systems and infrastructure for screening and triage in place?	- desktop and document reviews - stakeholder interviews	- service availability and readiness assessment indicators - harmonized health facility assessment indicator inventory - information from stakeholders at sub-district and PHC facility level (e.g. health facility managers, other health workers and CHWs) - IPC assessment reports		

Health system building block	#	Technical activity	Assessment questions	Data collection methods	Sources of data	Summary self-assessment	
						Assessment result*	Comments
	T12	Deployable outreach and mobile health service teams established to maintain access to care for displaced and affected populations while coordinating with CHWs and RCCE teams.	Are deployable outreach and mobile service delivery teams in place?	- desktop and document reviews - stakeholder interviews	- information from stakeholders at subnational, sub-district and PHC facility level (e.g. health facility managers, other health workers and CHWs) - job descriptions for outreach and mobile health service teams		
	T13	Reverse triage mechanisms in place to prioritise urgent cases during surge events while maintaining continuity of chronic (e.g. NCDs, TB/HIV, mental health) and essential care through PHC and community linkages.	Is a reverse triage mechanism to prioritize urgent cases in place?	- desktop and document reviews - stakeholder interviews	- health information system strategy/ policy/guideline documents - information from stakeholders at national, subnational, sub-district and PHC facility level (e.g. health facility managers, other health workers and CHWs)		
	T14	Remote consultation and telemedicine systems established in accordance with national regulatory frameworks and contextual needs.	Are mechanisms for remote consultation and telemedicine in place?	- desktop and document reviews - stakeholder interviews	- health information system strategy/ policy/ guideline documents - information from stakeholders at national, subnational, sub-district and PHC facility level (e.g. health facility managers, other health workers and CHWs)		
	T15	Contingency plans developed and operationalized to ensure PHC service continuity during acute emergencies, including arrangements for supply chains, human resources, infrastructure and technology.	Are contingency plans to ensure PHC service continuity during acute emergencies developed and operational?	- desktop and document reviews - stakeholder interviews	information from stakeholders at national, subnational, sub-district and PHC facility level (e.g. health facility managers, other health workers and CHWs)		

Health system building block	#	Technical activity	Assessment questions	Data collection methods	Sources of data	Summary self-assessment	
						Assessment result*	Comments
	T16	PHC emergency preparedness and service continuity plans developed and operationalized for multiple hazard scenarios, including continuity of essential services such as RMNCAH, NCDs and mental health.	Are PHC emergency preparedness and service continuity plans developed and operational?	- desktop and document reviews - stakeholder interviews	information from stakeholders at national, subnational, sub-district and PHC facility level (e.g. health facility managers, other health workers and CHWs)		
Medicines, medical products, vaccines and technologies (Supplies)	T17	Strategic stockpiles of essential medicines, supplies, personal protective equipment and medical countermeasures maintained at PHC facilities.	Are strategic stockpiles of essential medicines, supplies, PPE and medical countermeasures in place at PHC facilities?	- desktop and document reviews - stakeholder interviews	- information from stakeholders at subnational, sub-district and PHC facility level (e.g. health facility managers, other health workers and CHWs) - review of IPC assessment results (PHC)		
	T18	Forecasting and inventory management systems implemented to anticipate and manage needs for essential medicines and clinical supplies at PHC facilities.	Are forecasting and inventory management systems implemented?	- desktop and document reviews - stakeholder interviews	information from stakeholders at national, subnational, sub-district and PHC facility level (e.g. health facility managers, other health workers and CHWs)		
	T19	Resource pooling and coordination mechanisms across PHC networks established to maintain availability of essential medicines and supplies during disruptions.	Are resource pooling and coordination mechanisms across PHC networks implemented?	- desktop and document reviews - stakeholder interviews	information from stakeholders at subnational, sub-district and PHC facility level (e.g. health facility managers, other health workers and CHWs)		

Health system building block	#	Technical activity	Assessment questions	Data collection methods	Sources of data	Summary self-assessment	
						Assessment result*	Comments
Health information systems (Systems)	T20	Functional community health information systems in place to support patient care, follow-up and continuity of services, including through appropriate digital technologies and integration with national health information systems.	a. Are functional community health information systems in place to support patient care, follow-up and continuity of services? b. Are function community health information systems integrated with national health information systems?	- desktop and document reviews - stakeholder interviews	- information from stakeholders at national, subnational, sub-district and PHC facility level (e.g. health facility managers, other health workers and CHWs) - health information system strategy/ policy/ guideline documents		
	T21	PHC health information systems integrated with national health information systems support the collection, analysis and reporting of health service utilization data, including through digital medical record systems where feasible.	Is a functional HIS including use of digital approaches in place?	- desktop and document reviews - stakeholder interviews	- health information system strategy/ policy/guideline documents - service availability and readiness assessment indicators - harmonized health facility assessment indicator inventory - information from stakeholders at national, subnational, sub-district and PHC facility level (e.g. health facility managers, other health workers and CHWs)		

CHWs: community health worker; HHFA: harmonized health facility assessment; iCCM: integrated community case management; NCD: noncommunicable disease; PPE: personal protective equipment; RMNCAH: reproductive maternal newborn child and adolescent health; SOP: standard operating procedures.

* 1. not implemented, 2. partially implemented, 3. implemented, 4. fully implemented (with success / best practice)

Summary [treat]

Based on the scoring done above, please outline the key strengths of your program	
• What are the core gaps your programme needs to address? • What are the gaps identified through the country JEE/7-1-7 bottlenecks, IHR and Infection Prevention and Control Assessment Framework? • Why do these core gaps exist?	
• What are the critical priorities for your programme? • What are the key priority areas based on the gaps identified through the country JEE/7-1-7 bottlenecks, IHR and Infection Prevention and Control Assessment Framework? • What strategies will the country use to address identified priority areas? • Which stakeholders will you work with to address identified priorities?	

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